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C.1. Program Description

TRICARE is the Department of Defense (DoD) health care program administered by the Defense Health Agency (DHA) by means of the Military Health System (MHS) for approximately 9.5 million active duty and retired members of the Uniformed Services, and their spouses and children, including TRICARE for Life beneficiaries. The TRICARE Pharmacy Benefits Program is authorized under 10 U.S.C. §1074g and 32 C.F.R. 199.21.

The mission of the MHS is to enhance DoD readiness and national security by providing health support for the full range of military operations. The MHS must be prepared not only to provide a high quality, cost-effective health care benefit to its eligible beneficiaries during peacetime but also must be prepared to support the armed forces during exercises, contingencies, operations other than war, and in wartime. The MHS provides quality medical care through: (1) a network of health care providers and pharmacies in the United States and its territories; and (2) direct care Military Medical Treatment Facilities (MTFs) – (hospitals, clinics, and pharmacies) in the United States and overseas. The direct care system cannot support the total demand for health care services and is focused on maintaining the clinical skills of military staff to support medical readiness.

TRICARE augments the direct care system through a civilian network of providers and facilities serving its eligible beneficiaries. The TRICARE Pharmacy Benefits Program provides a world-class pharmacy benefit to all eligible beneficiaries through the integration of state-of-the-art technologies to enhance patient safety, efficiency, and cost-effectiveness. DoD administers an integrated TRICARE Pharmacy Benefits Program offering pharmacy services through MTFs, retail network pharmacies, retail non-network pharmacies, or delivery through the TRICARE Mail Order Pharmacy (TMOP). Retail network pharmacy services are currently available in all 50 states and the District of Columbia, Guam, Puerto Rico, and the U.S. Virgin Islands.

Features of the TRICARE Pharmacy Benefits Program include the use of the DoD Uniform Formulary, a tiered cost sharing structure, and a preference for generic over branded products. The DoD formulary is managed by the DoD Pharmacy and Therapeutics (P&T) Committee with input from the Beneficiary Advisory Panel (BAP) and final approval through DHA Director. This establishes the basic program benefits. Prescriptions for selected agents and supplies, hereafter referred to collectively as “agents” may be subject to prior authorization (PA) or utilization review requirements to assure Medical Necessity (MN), clinical appropriateness, and/or cost-effectiveness. There are statutorily required tiered cost-sharing by which beneficiaries partially defray costs of administering the pharmacy benefits program. Cost-sharing amounts differ based on the classification of an agent as generic, formulary, or non-formulary, in conjunction with the Point of Service (POS) from which the agent is acquired. The mail order and retail portions of this benefit are open to all eligible TRICARE beneficiaries.

Pharmacy benefits administrative functions under this contract include the following: performing claims adjudication, administering a retail pharmacy network, operating TMOP, process reimbursements for claims filled at retail network and non-network pharmacies, perform clinical reviews, and provide beneficiary and pharmacy support services. The Contractor shall transmit all claim information to the Government’s designated Pharmacy Data Warehouse (PDW).

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C.1.1. Overall Program Objectives

The following objectives identify the desired outcomes of this contract and are supported by the technical requirements in Section C:

- Efficient, accurate, and timely processing of claims and clinical review across all points of service, in accordance with the TRICARE benefit design, policy, regulation, and law.
- Convenient and timely beneficiary access to all agents covered by the TRICARE pharmacy benefits Program.
- Superior levels of agent care, safety, and beneficiary satisfaction.
- Effective management, quality controls, and oversight of all services and functions.
- Competitive cost management and optimized cost avoidance, cost containment, and use of Government resources.
- Delivery of the TRICARE Pharmacy Benefits Program through secure and reliable systems that promote seamless and streamlined operations for prescribers, beneficiaries, and other MHS stakeholders.

C.1.2. Definitions

Definitions specific to this contract, or not otherwise in Appendix A of the TRICARE Operations Manual, are provided in J-1.

C.1.3. Government Furnished Information

C.1.3.1. The Contractor shall interface with the Defense Enrollment Eligibility Reporting System (DEERS), according to the requirements established in the TRICARE Systems Manual.

C.1.3.2. The Government will provide licenses for the Contractor to access and use DEERS applications.

C.1.3.3. The Government will provide the Contractor with access to a medical pricing catalog which will be used for replenishment, at TMOP and for adjudicating federal electronic health record (EHR) claims.

C.1.3.4. The Government will provide initial Interface Control Documents (ICDs) and some additional technical specifications for the following systems:

- PDW
- Forensic Toxicology Drug Testing Laboratory Information Management System (FTDTL IMS)
- Immunization Tracking System
- Operational Medical Data Store (OMDS)
- Federal electronic health record

C.1.3.5. The Government will provide a quarterly data file for beneficiary mailings related to formulary changes, described under C.9.3.3.

C.1.3.6. The Government will provide a no less than quarterly file of beneficiary zip codes for use in evaluating and reporting on compliance with network access standards.

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C.1.3.7. The Director's Communications and Public Affairs Division of the Defense Health Agency (DCPAD) will design, develop, and print beneficiary educational materials, including written materials, briefings, and other methods of publicizing the TRICARE Pharmacy Benefits Program, excluding letters and other communication pieces required under this contract. The Government will provide an electronic portal where printed items can be ordered by the Contractor.

C.1.3.8. The Government will provide the PDW Data Dictionary and Data Schema, as described under C.11.8.

C.1.3.9. Before the start of pharmacy services, the Government will provide (via previous contractors) batch files containing all retail, mail, and MTF claims along with PA and MN determinations for the past two-year period. The Government (via the outgoing contractor) will also provide an Other Health Insurance (OHI) data file.

C.1.4. Requirements Documents

C.1.4.1. The following documents are hereby incorporated by reference and form an integral part of this contract. Documentation incorporated into this contract by reference has the same force and effect as set forth in full text.

- Title 10, United States Code, Chapter 55
- 32 CFR Part 199
- TRICARE Policy Manual (TPM) 6010.63-M, April 2021
- TRICARE Reimbursement Manual (TRM) 6010.64-M, April 2021
- TRICARE Systems Manual (TSM) 7950.43-M, April 2021
- TRICARE Operations Manual (TOM) 6010.62-M, April 2021

When changes are made to the above statutes or regulations, they are automatically incorporated into these contract requirements.

C.1.4.2. TRICARE Manuals.

C.1.4.2.1. The TRICARE manuals provide instruction, guidance, and responsibilities in addition to the requirements set forth in the incorporated federal statutes and regulations listed in C.1.4.1 and may not be interpreted in contradiction thereto.

C.1.4.2.2. The Contractor shall utilize the current version of the TRICARE Manuals T-5, published at Attachment J-3, line 11. The manuals are applicable in their entirety.

C.1.4.2.3. The Contractor shall routinely review proposed, postponed, and published manual changes and discuss concerns with the Contracting Officer Representative (COR).

C.1.4.2.4. When a new manual change is postponing a publication (identified with a change number and date), the COR and Contractor will coordinate to confirm the applicability of a change to the contract.

C.1.4.2.5. To the extent practicable, for those manual changes that apply to the contract, the Government

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and Contractor will negotiate in good faith a bilateral agreement that incorporates the change into the contract. This in no way precludes the Government's exercise of its rights to execute unilateral change orders as circumstances may require.

C.1.4.2.6. Manual changes that do not apply to the current contract will be incorporated into the next modification of the contract.

C.1.4.2.7. Among the manuals, the TRICARE Policy Manual (TPM) takes precedence over the other three TRICARE Manuals. The TRICARE Reimbursement Manual (TRM) takes precedence over the TRICARE Systems Manual (TSM) and the TRICARE Operations Manual (TOM). The TSM takes precedence over the TOM. In the event of conflict between language found within the TRICARE manuals and the contract, the contract prevails.

C.2. General Claims Processing.

C.2.1. The Contractor shall accept and process claims submitted by retail network pharmacies, TMOP, MTF pharmacies, Veteran Affairs (VA) pharmacies, Indian Health Services (IHS) pharmacies, beneficiaries for direct reimbursement (including non-network), and batch files submitted by State Medicaid Agencies.

C.2.2. Unless stated otherwise, the Contractor shall include claims adjudication processes outlined in C.2, consisting of eligibility check, application of the correct cost-sharing amount, identification of OHI, benefit design edits, Drug Utilization Review (DUR) and application of catastrophic cap and deductible updates. The Contractor shall maintain a complete patient profile, inclusive of all claims processed within the scope of this contract. The Contractor shall maintain a complete patient profile, inclusive of all claims processed within the scope of this contract.

C.2.3. Appeal Rights. The Contractor shall not accept appeals from Active Duty Service Members (ADSMs). ADSMs are not authorized as appealing parties for TRICARE cost-sharing determinations. ADSMs should be directed to their local MTF for review of access to care issues.

C.2.4. The Contractor may, as a courtesy to beneficiaries, perform internal coordination of benefits between TMOP and contractor-administered OHI to prevent the beneficiary from having to submit a separate reimbursement claim.

C.2.4.1. If the Contractor receives a prescription or supporting documentation (e.g., reimbursement claim, PA request) for a beneficiary with Contractor-administered OHI, the Contractor shall internally transfer to its commercial operation in lieu of rejecting or returning to source.

C.2.4.2. If the Contractor receives a prescription or supporting documentation through its commercial operational process for a beneficiary with TRICARE, the Contractor shall process the commercial as primary and automatically submit the secondary claim to TRICARE, billing the beneficiary for any amount remaining after the TRICARE secondary coverage is applied.

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C.2.5. Claims Processing System

C.2.5.1. The Contractor shall provide 24 hours a day, 7 days a week claims processing for all locations, including overseas MTFs.

C.2.5.2. The Contractor's claims processing system shall be available no less than 99.5% of the time, excluding external interface partner downtime. The system is unavailable when the rate of certain claims rejects exceeds 10% for at least 30 minutes. These rejects include those tied to system non-availability, based on mutual agreement with the Government, and documented in CDRL M060. Contractor downtime concurrent with external system downtime is excluded from the system availability calculation.

C.2.5.3. The Contractor shall provide reporting on system processing performance per Contract Data Requirements List (CDRL) M060. At the request of the Government, the Contractor shall provide documentation of reported downtime, including detailed explanation of the causes.

C.2.5.4. The Contractor shall immediately communicate unscheduled downtimes of no fewer than 15 minutes to MTF pharmacy staff and other recipients identified by the Government. System availability updates will be provided at a Government determined interval and will include an estimate of when the system will once again be available for use. A final notification shall take place to inform users that regular processing has been restored. The communication mechanism shall be provided and maintained by the Contractor.

C.2.5.4.1. The Contractor shall schedule maintenance windows, to the greatest extent possible, to coincide or overlap with Defense Manpower Data Center (DMDC) maintenance windows, as described in TSM Chapter 3, Section 4.2. The TSM reflects a maximum window within which DMDC scheduled maintenance will occur. Actual planned duration is communicated by DMDC on a weekly basis (See C.11.5.2.). DMDC will attempt to accommodate the Contractor's needs when establishing a maintenance schedule.

C.2.5.5. The Contractor shall notify the Government no less than 72 hours in advance of any planned maintenance window outside the DMDC maintenance window. The Contractor shall also send a final notification to inform users that regular processing has been restored. Such notifications shall include MTF pharmacy staff and other recipients identified by the Government, through the process under C.2.5.4 .

C.2.5.6. The Contractor shall accept any one of the multiple identifiers that may be submitted to process claims. Identifiers include but are not limited to DoD ID, social security number (SSN), or DoD Benefits Number (DBN). The primary identifier used by the Contractor shall be the DoD ID, as described in the TSM, and the Contractor's system shall link the identifier transmitted by the pharmacy with the DoD ID. The Contractor shall dynamically link all variations of patient IDs to ensure a single patient profile, including instances where patients may be eligible under health plans because of having more than one sponsor.

C.2.5.7. The Contractor shall ensure that their system utilizes software which maximizes configurable parameters to allow for changes to be made quickly and efficiently.

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C.2.6. DEERS

C.2.6.1. Verifying Eligibility. The Contractor shall not authorize payment for a prescription prior to verifying eligibility, except at the direction of the Government.

C.2.6.1.1. In some cases, the Government may authorize the Contractor to process one or multiple claims regardless of DEERS eligibility or date of service. Any necessary overrides must be available to allow for real-time electronic claims and paper claims to process at any pharmacy point of service.

C.2.6.2. The Contractor shall minimize queries to DEERS that are not required for the purpose of authorizing payment or obtaining demographic data to correctly apply benefit design rules.

C.2.6.2.1. The Contractor shall monitor DEERS query volumes for all interfaces throughout the period of performance against the projections provided during transition. The Contractor shall investigate the variance in these volumes and revise estimates for future periods of performance annually, or any time that the actual query volume(s) deviates more than 10% from the most recent projections (CDRL A090).

C.2.6.2.2. The Contractor shall optimize their system to reduce timed-out transactions to DEERs.

C.2.6.3. Catastrophic Cap and Deductible (CC&D)

C.2.6.3.1. The Contractor shall use the beneficiary's CC&D status to apply the correct cost-sharing amount and deductible and perform updates in accordance with TOM Chapter 23, Section 3. Separate catastrophic caps are maintained for Continued Health Care Benefits Program (CHCBP), in accordance with TOM Chapter 23, Section 3.

C.2.6.3.1.1 The Contractor shall receive and provide reporting on the maintenance of CHCBP catastrophic caps (CDRL Q120)

C.2.7. Claims Processing Edits

C.2.7.1. The Contractor shall support claims adjudication rules specific to the POS.

C.2.7.2. The Contractor shall apply comprehensive edits at the point of dispensing in accordance with the appropriate benefit design in force for that date and POS and commercial best practices. New claims shall be screened against the complete patient profile (see C.2.3). The Contractor shall return appropriate clinical warnings or administrative alerts.

C.2.7.3. The Contractor shall perform real-time edits that may be specific to DoD and fall outside standard commercial practice, including but not limited to Prescription Monitoring Program, sanctioned providers, safety reviews established through the Uniform Formulary process, prescriber status, DEERs data (e.g., beneficiary category, AD status), and the ability to suppress real-time edits by POS based on Government direction.

C.2.7.4. The Contractor shall be responsible for processing claims with dates of service prior to the start of this contract. This includes initial submission of claims, and any adjustments, corrections, cancellations or recoupments. The Contractor shall perform all necessary research to process the

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claim according to benefit design and formulary restrictions in effect on the date of service.

C.2.7.5. At the Government's direction, the Contractor shall process any claim or prescription order. The Contractor shall apply any required overrides to allow the prescription to process successfully, resulting in a "paid" status.

C.2.7.6. State of Emergency (SOE) Declarations

C.2.7.6.1. The Contractor shall monitor SOE declarations issued by Federal and State Governments and provide a recommendation start and end date for the implementation of emergency refill too soon procedures. The recommendation will include the start and end date of the SOE as provided by the Governor, state, or issuing agency. The Contractor's recommended start date should coincide with the date when the recommendation was made to the Government.

Upon approval by the Government, the Contractor shall have the capability to bypass the refill too soon edit and allow the refill to be processed for areas covered by the SOE. The Government may also direct the Contractor to implement these measures for specific areas outside of a declared SOE.

C.2.7.6.2. The TMOP shall not ship agents to areas under a declared emergency without confirming that both delivery by carrier and receipt by beneficiary are possible. For packages shipped prior to the emergency declaration that were reported lost or missing, the Contractor shall immediately ship the replacement to the beneficiary's alternate address and refund copay for the lost shipment upon request per C.6.2.11 and C.6.2.12

C.2.7.6.3. The Contractor shall provide standard emergency operating procedures which address the following:

- Avoid waste when shipping to areas where there is disaster, including holding shipments, confirming carrier delivery availability, and verifying current beneficiary location.
- Facilitate beneficiary access to agents during a declared emergency at both retail and mail POS, including but not limited to bypassing edits, replacing lost shipments, and shipping to alternate addresses. Facilitating access at MTF POS will be limited to bypass of edits.
- Maintain communication channels to keep stakeholders informed, including:
 - Beneficiaries: how they can receive their agents, in coordination with DCPAD.
 - MTFs: possible patient movement, use override codes, and how to transfer prescriptions
 - Retail Network: possible patient influx (e.g., local MTF being incapacitated)
 - DHA leadership: continual monitoring of SOE and recommendations for extension or early termination of emergency operation procedures
- Identify pharmacies impacted and advise of open network pharmacies within the impacted area

C.2.7.6.4. The Contractor shall document policies and processes for SOE declarations in playbooks and make available to the Government up request.

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C.2.7.6.5. After an event, the Contractor shall provide monthly reporting on the overrides that were granted under the Emergency procedures (CDRL R070). An event may be comprised of several individual Emergency Declarations (i.e., a hurricane that impacts several counties and states).

C.2.7.6.6. The Contractor shall not send off hours SOE requests to the Government unless prepared to act upon the response on the same day it is received during off hours.

C.2.8. Other Health Insurance (OHI)

C.2.8.1. The Contractor's OHI file shall be the system of record for OHI. The Contractor shall utilize all possible data sources to maximize the identification of OHI, including but not limited to leads provided by the following:

- Managed Care Support Contractors (MCSC)
- MTF COB claims
- Agent manufacturers
- Claim forms
- Beneficiary declarations
- Contractor's internal files
- Commercial services or data sources

C.2.8.2. The Contractor shall develop OHI records in accordance with TOM, Chapter 23, Section 3, TOM Chapter 10, Section 4, and TRM, Chapter 4.

C.2.8.3. If OHI information between sources is inconsistent, the Contractor shall utilize logic to determine which source is the most reliable in a quick and efficient manner.

C.2.8.3.1. The DEERS eligibility system will provide an indicator of Medicare Part D (MPD) enrollment. The Contractor may utilize this information or use an alternative approved source for determining MPD coverage status.

C.2.8.3.2. The Contractor shall track and mitigate discrepancies between DEERS MPD information or alternative MPD source (see C.2.9.3.1) and information conveyed by the beneficiary. If a beneficiary notifies the Contractor that their MPD status is not correct in the system, the Contractor shall have a process to document the correct MPD status and adjudicate claims based on the updated status. Unless there is a request for reimbursement, the Contractor shall accept the beneficiary's attestation of coverage or lack of coverage.

C.2.8.3.3. In cases where DEERS or alternative MPD source did not reflect MPD coverage that was in place, the Contractor shall also provide reimbursement for past claims where the patient's copay exceeded what is allowable with MPD. The Contractor shall ask for appropriate documentation to verify coverage dates. For reimbursement request of claims dated prior to 1 Jan 2023, the Contractor shall request approval from the Government.

C.2.8.3.4. The Contractor shall use the Processor Control Number (PCN) submitted on each incoming COB claim. If the MPD PCN is used on the claim submission, the Contractor shall treat the incoming claim as an MPD claim in all respects. The submitted PCN shall take precedence over all other data sources.

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C.2.8.4. When new OHI is identified, the Contractor shall pursue recoupment for past claims and build out the beneficiary's profile for future claims. (See C.2.8.4.) Post-adjudication, the Contractor shall perform reviews to ensure the claim processed correctly and update their system as necessary.

C.2.8.5. The Contractor shall have a mechanism to allow a DEERS OHI record to be bypassed for up to one year based on the beneficiary's attestation that such OHI does not exist. Claims within that time will continue to process without rejecting for the reported OHI record.

C.2.8.6. The Contractor shall document their policies for the application of OHI information in claims processing, including the use of different types of DEERS records and how discrepancies are resolved. This documentation will be made available for the Government's review upon request.

C.2.8.7. The Contractor shall provide reporting to the Government on OHI development (CDRL Q110) and cost avoidance (CDRL Q111).

C.2.9. Claims Processing System Documentation

C.2.9.1. The Contractor shall develop and maintain comprehensive documentation of all aspects of claims processing not covered in the benefit design document (see C.2.9.4), including but not limited to OHI/COB, Medicare Part D Claims, pricing logic, thresholds for specific edits (eg, refill too soon), rules specific to a point of service (including MTF Set-Up Document – see C.5.4), and any other claims adjudication rules currently in use that are not defined by the DoD P&T or by law. This combined documentation shall allow the Government to correctly determine how any given claims processing scenario will adjudicate. Updated versions of any of these documents shall be made available in accordance with C.14.13.

C.2.9.1.1. Current documentation shall be made available to the Government prior to Benchmark Testing during transition and be used to evaluate results.

C.2.9.2. The Contractor shall maintain all payer sheets used in the transmission of DoD claims from all POS.

C.2.9.3. The Contractor shall maintain a benefit design document and provide the Government with access to the current version via a web-based platform. The Contractor's presentation of the benefit design within this document shall remain consistent with the elements of the document initially provided by the Government and shall be in a format agreed to by the Government. The benefit design shall contain formulary and benefit restrictions based on beneficiary category, enrollment category, POS, agent by category, law and regulation, and any other factors. The benefit design document will include the following lists:

- MTF Basic and Extended Core Formularies
- Compound inclusions and exclusions
- Covered over the counter (OTCs) to include agents specifically covered for the federal EHR
- Federal Ceiling Price (FCP) Non-Compliant Retail Exclusions
- Non-Formulary Agents
- PA
- Step-Therapy Agents

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- Self-Administered Injectables
- Smoking Cessation Agents
- Specialty Agents
- Covered Vaccines
- Non-covered Agents
- Quantity Limit (QL) Restrictions
- Age restrictions
- Expanded Mail Order and MTF drug list (specialty and non-specialty)
- Agents Cost Exceed Max List

C.3. Retail Prescriptions

C.3.1. Retail Prescription Claims

C.3.1.1. The Contractor shall accept and process all claims for agents and supplies covered under the TRICARE Pharmacy Benefits Program and purchased from a licensed pharmacy in the 50 United States, the District of Columbia, Puerto Rico, the U.S. Virgin Islands, Northern Mariana Islands, American Samoa and Guam.

C.3.1.2. The Contractor shall limit retail claims for covered agents under 10 U.S.C. §1074g, 32.C.F.R. 199.21, and other applicable regulations. Network pharmacies may submit claims for covered supply items using the National Drug Code (NDC) numbers assigned to them. Reimbursement of retail pharmacy claims will be made in accordance with G.3.6.2.

C.3.1.4. When access to specific agents at retail pharmacies is restricted under the law, 32 CFR 199.21 (r), the Contractor shall facilitate either a change in the beneficiary's current prescription to an approved agent or shift the beneficiary's prescription to TMOP. Covered agents that are restricted at retail and their respective implementation dates are published at in DoD P&T Committee Meeting Minutes (Attachment J-3, Line 1)

C.3.1.5. When claims are received that are out of jurisdiction for the TRICARE Pharmacy Contract, the Contractor shall forward them to the TRICARE Contractor with jurisdiction for those claims, in accordance with TOM, Chapter 8, Section 2 and TOM Chapter 23, Section 2.

C.3.1.6. The Contractor shall complete real-time, online Coordination of Benefits (COB) in accordance with National Council of Prescription Drug Program (NCPDP) F6 standards (or most current version) for those claims filled in retail network pharmacies where OHI has been identified, to include Medicare Part D claims. The Government will provide the COB and Medicare Part D billing transaction segments to include the required values. The Contractor is required to track Medicare Part D True Out-Of-Pocket expenses (TROOP) and total agent expenditures for each TRICARE beneficiary who is also enrolled in Part D. The Contractor shall provide this information to the Centers for Medicare & Medicaid Services (CMS) designated TROOP facilitator.

C.3.1.7. The Contractor shall reimburse claims in accordance with the TRM, Chapter 4.

C.3.1.8. The Contractor shall process claims submitted by Department of VA pharmacies for beneficiaries who are TRICARE-eligible. These claims shall with processed with TRICARE as primary payer using business rules provided by the Government. VA benefits are not considered

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OHI. The Government will provide a list of VA pharmacies to the Contractor. VA benefits are not considered OHI.

1. The Contractor shall determine cost-shares for beneficiaries with Medicare Part D in accordance with TOM Chapter 23, Section 3.

C.3.1.9. If requested by the beneficiary and allowable under federal and state law, the Contractor may authorize the dispensing of up to a 90-day supply prescription as a single transaction at a retail pharmacy. In these cases, the pharmacy shall collect a copay for each 30-day increment. The Contractor must make this option available at all retail network pharmacies.

C.3.1.10. The Contractor shall ensure that claims for prescriptions filled but not dispensed are reversed within ten (10) calendar days of the date the original claim was submitted. When reversals are processed more than ten (10) calendar days after the date the original claim was submitted, the Contractor shall adjust or cancel the TRICARE Encounter Data (TED) record.

C.3.1.11. The Contractor shall process batch claims in the most current NCPDP batch format. The Contractor may receive batch claims from a variety of sources (e.g., State Medicaid agencies, clearinghouses) and the Contractor shall process these claims regardless of the electronic media (e.g., CD ROM, tapes) through which they are submitted. The Contractor shall process claims from state agencies in accordance with TRM Chapter 1 Section 20. All batch claims shall be processed within two business days of receipt.

C.3.1.12. The Contractor shall report the actual amount paid to the retail pharmacy. On the TED, this will be populated as the Amount Paid by Government Contractor, per the TSM, Chapter 2.

C.3.2. Reimbursement Claims

C.3.2.1. The Contractor shall process reimbursement claims submitted by beneficiaries, also known as Direct Member Reimbursement (DMR) claims, and those submitted by pharmacies, known as assignment of benefit claims, in accordance with TOM, Chapter 23, Section 3, Paragraph 1.2 and Chapter 8, Section 1, Paragraph 3.1. The Contractor shall accept claims submitted using any of the specified forms. Electronic submission of reimbursement claims is only allowed under an approved Office of Management and Budget (OMB) format.

C.3.2.2. Upon request, the Contractor shall mail the current version of the DD2642 claim form to beneficiaries.

C.3.2.3. The Contractor shall process reimbursement claims for non-network pharmacy services in accordance with the TRM, Chapter 1, Section 15, minus applicable cost-sharing amounts and deductibles.

C.3.2.4. The Contractor shall process these reimbursement claims using the most current NCPDP format.

C.3.2.5. The Contractor shall monitor reimbursement claims processing and work with retail network pharmacies to reduce the volume of network reimbursement claims. Network pharmacies should be using online coordination of benefits in most cases. The Contractor shall work with pharmacies with higher-than-average rates of reimbursement claims to transition

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them to online processing.

C.3.2.6. The Contractor shall not report or bill manual corrections to retail pharmacy claims as reimbursement claims.

C.3.2.7. The Contractor shall process claims for beneficiaries who are required by their OHI to use their designated mail order pharmacy or other non-network pharmacy using network cost shares. Non-network cost-sharing amounts and deductibles are not applicable to these claims.

C.3.2.8. The Contractor shall routinely audit reimbursement claims to look for scenarios such as high-cost claims, high cumulative claims value, and high volume of claims. All such reviews and audits shall be performed at the beneficiary and family level.

C.3.2.9. Measured monthly, reimbursement claims shall meet the following minimum standards:

C.3.2.9.1. 95% of reimbursement claims shall be processed to completion within 10 calendar days of receipt.

C.3.2.9.2. 100% of reimbursement claims shall be processed to completion within 14 calendar days of receipt.

C.3.2.10. The Contractor shall provide reporting on reimbursement claims volumes, processing times, denials and appeals (CDRL Q050). Reimbursement claims are considered processed to completion as of their TED record Create Date.

C.3.2.11. The Contractor shall provide notification to the beneficiary in writing for denied reimbursement claims. The notification must explain why the claims were denied and detail the beneficiary's appeal rights.

C.3.2.12. Under the TRICARE Pharmacy benefits Program, the Contractor shall not process reimbursement claims for prescriptions filled at MTF pharmacies. If necessary, the Contractor may forward the claims to its commercial services section for review as a claim payable by a commercial insurance plan.

C.3.3. Retail Pharmacy Network

C.3.3.1. The Contractor shall establish and maintain a retail pharmacy network throughout the 50 United States, the District of Columbia, Puerto Rico, the U.S. Virgin Islands, and Guam. The Contractor shall provide network retail pharmacy services in American Samoa and the Northern Mariana Islands when they become eligible.

C.3.3.2. Retail Network Access. The Contractor's retail pharmacy network shall meet the following:

C.3.3.2.1. Access Requirements, as measured by beneficiary zip code:

C.3.3.2.1.1 At least one network pharmacy within 15 minutes driving time of 90% of the beneficiaries;

C.3.3.2.1.2 75% of the 10% of beneficiaries that do not have a network pharmacy within 15 minutes driving time shall have at least one pharmacy within 30 minutes driving time.

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C.3.3.2.2. No fewer than 35,000 retail network pharmacies.

C.3.3.2.3. Exclusions

C.3.3.2.3.1. MTF, VA, Public Health Service (PHS), and Indian Health Service (IHS) pharmacies shall not be considered as retail network pharmacies for purposes of this requirement.

C.3.3.2.3.2. Beneficiaries residing on military installations shall be excluded from this metric. The Government will provide a list of zip codes of military installations for the Contractor's use in calculating this metric.

C.3.3.3. The Contractor shall use commercially available software or web tools to calculate this metric. The methodology shall be made available to the Government for audit upon request.

C.3.3.4. The Contractor shall provide reports identifying retail network pharmacies and the total network size (CDRL M040), network access relative to the above metrics (CDRL M041), and network pharmacy performance (Q190).

C.3.3.5. The Contractor shall provide a plan describing how beneficiaries not meeting the access standards will have access to pharmacy services. (CDRL A110)

C.3.3.6. All network pharmacies shall be fully licensed in accordance with applicable Federal and State laws and have a current NCPDP number. Retail pharmacies who offer to mail prescriptions to beneficiaries as part of their business may be included in the network subject to retail pharmacy specifications listed herein.

C.3.3.7. To the extent possible allowed by Federal law and regulation and this contract, retail pharmacies shall provide TRICARE beneficiaries the same type, level, and quality of service provided to beneficiaries of other commercial clients.

C.3.3.8. The Contractor shall ensure that all pharmacies document the receipt of the agent by the beneficiary or the individual authorized by the beneficiary, in accordance with all applicable State and Federal Laws. The Contractor shall ensure that network pharmacies have procedures to reasonably assess the validity of prescriptions ordered by telephone.

C.3.3.8.1. The Contractor shall require all retail network pharmacies to utilize online Coordination of Benefits.

C.3.3.9. Changes to the Retail Pharmacy Network

The Contractor shall have a plan for communicating to beneficiaries when a pharmacy is removed from the network. As part of the plan, the Contractor shall do the following in the event of network changes:

- Provide the Government with the names of all pharmacies selected for removal from the network at a minimum of 120 days prior to the effective date of the changes.
- Identify and provide advance notification to beneficiaries who have filled prescriptions at the designated pharmacies during the previous six (6) months. The notification shall provide the beneficiary with names and addresses of the pharmacies closest to the beneficiary remaining in the network. The notification shall include information on how

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to search for additional pharmacy options. The Contractor shall ensure that the beneficiary receives the letter at a minimum of 30 days prior to the effective date of the change.

- Provide the Government with samples of all beneficiary correspondence related to the change in the network for comment. The Government shall have no less than 14 days to review and approve.
- Monitor and track prescription fills by impacted beneficiaries in specific categories (CDRL R020).
- Make changes to the plan as necessary to ensure successful communication with all beneficiaries and minimum disruption to therapy.

C.3.3.9.1. In situations where a pharmacy fails to meet credentialing standards or there are indications of potential fraud that does not meet the standard requirements for case development per TOM Chapter 13, the Contractor may remove the pharmacy from the network without providing advance notification to the Government or beneficiaries. For other situations that justify short-notice removal of a pharmacy, the Contractor shall contact the Government for approval. The Contractor shall notify the impacted beneficiaries within 14 days after the pharmacy is removed.

C.4. Specialty Agents and Clinical Services

C.4.1. The Government will provide an initial list of agents considered specialty agents. The Contractor shall propose updates to the list based on changes in the market, evolution of clinical practices, limited access, cost effectiveness, clinical support considerations, and overall benefit to the TRICARE Pharmacy Benefits Program for Government approval. The Contractor shall provide the methodology under which agents are evaluated and proposed for the addition or removal from the specialty agent list.

C.4.2. Network Pharmacies Capable of Dispensing Specialty Agents. All network pharmacies that can dispense specialty agents shall be fully licensed in accordance with applicable Federal and State laws and have a current NCPDP number. When not dispensed by an MTF pharmacy or a VA pharmacy, all covered specialty agents, including limited distribution agents, shall be accessible to beneficiaries at network retail cost shares or mail order shares if the TMOP dispenses the specialty agent.

C.4.3. The Contractor shall not deny cost-sharing or reimbursement for claims associated with specialty agents filled at network pharmacies unless the specialty agent is enrolled in Expanded Use of MTF and TMOP (C.6.6), subject to benefit design restrictions (C.7.3.1) or agents non-compliant with, 10 USC 1074g(f) as implemented by 32 C.F.R. 199.21(q).

C.4.4. The Contractor shall provide notification to the Government of any changes to the network per C.3.3.9.

C.4.5. Access. The Contractor shall ensure all specialty agents as defined by C.4.1 are available to TRICARE beneficiaries in the pharmacy network. If requested by the Government, the Contractor shall facilitate the movement of specialty prescriptions from MTF pharmacies to TMOP or other network pharmacies.

C.4.6. The Contractor shall provide education on obtaining specialty agents through the network

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pharmacies if requested by beneficiaries and/or providers. This shall include but is not limited to:

- Education on obtaining agents that have access restrictions (e.g. limited distribution agents).
- Assistance in transferring prescriptions or obtaining new prescriptions.
- Education on all options in the pharmacy network for obtaining a prescription for specialty agents and the benefits of these options within the pharmacy network to allow beneficiaries and providers to make an informed choice.
- Assist in a waiver request if appropriate per C.6.6.
- On-boarding and registration activities at network pharmacies, when appropriate, that can support the dispensing of specialty agent.

C.4.7. Patient Support. The Contractor shall ensure beneficiaries who receive specialty agents from network pharmacies are provided support in accordance with pharmacy best practice standards. At TMOP and Contractor selected retail network pharmacies (if applicable), the Contractor shall provide agent administration support (e.g. patient consultation, therapy evaluation and call center support) that minimizes adverse events. The Contractor shall maximize adherence as appropriate, promote overall well-being and a high level of beneficiary satisfaction as measured by widely recognized industry benchmarks.

C.4.8. Patient Support. The Contractor shall ensure beneficiaries who receive specialty agents from network pharmacies are provided support in accordance with pharmacy best practice standards. At TMOP and Contractor selected retail network pharmacies (if applicable), the Contractor shall provide agent administration support (e.g. patient consultation, therapy evaluation and call center support) that minimizes adverse events. The Contractor shall maximize adherence as appropriate, promote overall well-being and a high level of beneficiary satisfaction as measured by widely recognized industry benchmarks.

C.4.9. The Contractor shall educate beneficiaries on manufacturer supported programs that may augment, but not replace, those services offered by the network pharmacies.

C.4.9.1. At TMOP and Contractor selected retail network pharmacies (if applicable), the Contractor shall provide delivery management services to include timely delivery of agents to align with treatment schedules, delivery tracking and waste prevention.

C.4.9.1.1. The Contractor shall report, per C.6.1.11, the following specialty prescription processing time metrics, specific for TRICARE beneficiaries, for both TMOP prescriptions and Contractor selected retail network pharmacies (if applicable) per CDRL Q040.

C.4.9.2. The Contractor shall ensure beneficiaries receive accompanying supplies for specialty agents dispensed at TMOP and Contractor selected retail network pharmacies (if applicable), including those required for administration, disposal or any other purposes in accordance with pharmacy best practices.

C.4.9.3. At TMOP and Contractor selected retail network pharmacies (if applicable), the Contractor shall work with the MCSC(s) to coordinate agent administration when appropriate (C.15.1.16.4). If required, this shall include the administration or disposal of agents, healthcare provider support or other available services as deemed appropriate.

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C.4.9.4. Programmatic Support. The Contractor shall optimize utilization strategies as they relate to C.7.2.1 and the incorporation of industry best practices that align with the TRICARE benefit design. The Contractor shall optimize cost controls.

C.4.9.5. Replenishment at TMOP. The Contractor shall maximize replenishment of specialty agents available from the National Prime Vendor (NPV). The Defense Logistics Agency-Troop Support (DLA-TS) manages the NPV contract (C.6.8.9).

C.4.9.6. Replenishment will be owed to the Contractor when all of the following conditions are met: 1) specialty agents are dispensed from the TMOP to TRICARE eligible beneficiaries; 2) specialty agents are available for replenishment from the NPV; and 3) specialty agents are not acquired through the Market Priced Pharmaceutical Program per C.4.7.6.

C.4.9.7. The Government will work with the Contractor to determine which non-specialty agents are eligible for replenishment at pharmacies designated as replenishment pharmacies to dispense specialty agents per C.6.1. Non-specialty agents determined to be replenishable by the Government will follow the mail order requirements in C.6 and G.3.5.7.3.

C.4.9.8. The Contractor shall request replenishment and follow the requirements in accordance with C.6.8.

C.4.9.9. Non-specialty and specialty agents which are not replenished from the NPV will follow retail prescriptions claims processing per C.3.1.

C.4.9.9.1. The Contractor shall identify its preferred Returns Management Reverse Distributor (Reverse Distributor) to the Government and further distinguish between returns for which replacement of an agent is expected from the NPV per C.6.4.

C.4.9.10. For non-replenished specialty agents, the costs will be reimbursed (see Section G.3.6.1). Non-replenished agents are subject to cost control measures under the retail network specialty guarantee (See Section H.3.3). In instances where a specialty agent is not replenished in a timely manner, as agreed upon by the Government and the Contractor, the Contractor shall coordinate with the Government to determine the best course of action for continued efforts towards replenishment of specialty agents. This may include but is not limited to: (1) the use of the Contractor's inventory until replenishment is available through the NPV (C.6.8.13); or (2) utilizing the Market Price Pharmaceutical Program (C.4.7.6 and C.6.8.22). Any replenishment owed at the end of each option period will be reconciled per C.6.8.7.

C.4.9.10.1. If a specialty agent becomes non-replenished from a previous replenished status, the Contractor will provide education to beneficiaries and/or providers on where the agent can be accessed and whether benefit restrictions or co-pay changes may occur.

C.4.9.10.2. If a specialty agent is unavailable at TMOP or the Contractor selected retail network pharmacies (if applicable), the Contractor shall ensure the beneficiary is informed of the situation and will educate the beneficiary and/or provider of their options in obtaining the agent, as well as complete the transfer of that prescription. If appropriate, the Contractor shall work with the beneficiary's provider to find alternative therapies which may be available at TMOP, or the Contractor selected retail network pharmacies (if applicable).

C.4.9.11. Reporting. The Contractor shall provide a report on the dispensing of specialty

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agents and clinical services provided (CDRL Q150).

C.5. MTF Pharmacy Claims

C.5.1. The Contractor shall support claims submissions from MTF Pharmacies using the federal EHR.

C.5.2. The Contractor shall connect to sites using the Federal EHR as described in Section C.11.7.

C.5.3. The Federal EHR is a commercial standard EHR and processes claims in NCPDP D.0 format (or the most current standard). It is supported by the Defense Healthcare Management System Modernization (DHMSM) contractor.

C.5.4. The Contractor shall review any changes to claims processing parameters with the Government before they are implemented, including any NCPDP mandated changes to the transaction format such as changes to reject codes and overrides.

C.5.5. The Contractor shall process claims using business rules for the Federal EHR claims, which include the benefit design (including P&T guidance described in C.6.8.22.7) and other documentation (e.g., MTF Set-Up). The Government may direct specific changes to the Federal EHR business rules as appropriate to support mission readiness. The contractor will support pre-deployment activities as it pertains to the various Combatant Commands the deployment falls under per C.6.15.6. up to as outlined in Area of Responsibility (AOR) Mods, such as USCENTCOM MOD).

C.5.6. The Contractor shall support coordination of benefits for the Federal EHR claims, in accordance with the MTF Edit Set-Up document

C.5.7. The Contractor shall support two unique processor control numbers (PCN)s for the Federal EHR claims.

C.5.8. The Contractor shall consult its internal OHI database when processing MTF claims. If the patient is identified as having OHI, the Contractor shall return a reject that is overridable at the MTF's discretion, without any action by the beneficiary or contacting the MTF Help Desk.

C.5.9. The Contractor shall not enforce eligibility for MTF claims. The MTF assumes responsibility for eligibility; therefore, the Contractor will process the claim regardless of eligibility status.

C.5.10. The Contractor shall not perform CC&D updates for MTF claims.

C.5.11. The Contractor shall maintain current, accurate, and complete system documentation, as described in C.2.9.

C.5.12. The Contractor shall coordinate an interface control document (ICD) with the DHMSM Contractor. The ICD will be approved by the Government.

C.5.13. Federal EHR claims will utilize a full range of commercial edits with some exceptions, as defined here and in the Edit Set-Up documents. All transactions shall be transmitted to the PDW.

C.5.14. Claims received under C.11.7.1.4 shall be excluded from edits, with no rejects applied.

C.5.15. The Contractor shall adjudicate all Federal EHR claims and shall reject claims that do not pass

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edits. The Contractor may return a paid claim with advisory messaging in lieu of rejection or exclude specific agents from such rejects as directed by the Government. The Government may identify specific information to be included in the messaging for advisories or rejects. Commercial standard edits shall apply unless otherwise indicated.

C.5.16. The Contractor shall accept the ingredient cost submitted on the claim for the Federal EHR claims. No adjustments or discounts shall be applied.

C.6. Mail Order Pharmacy

C.6.1. Mail Order Pharmacy Prescriptions. The TMOP program, as administered under the terms of this contract, is the Contractor's Mail Order Pharmacy(s) which supports the dispensing of TRICARE covered non-specialty and TRICARE designated specialty agents. These pharmacies shall receive replenishment inventory from the NPV. All requirements for administration of the TMOP under this contract shall be applicable to the dispensing of non-specialty and specialty agents unless specified otherwise.

C.6.1.1. The Contractor's mail order pharmacy shall dispense and deliver agents to TRICARE beneficiaries consistent with the requirements that apply to the overall standard operations of TMOP outlined below. For beneficiaries in deployed theaters of operation, the Contractor shall dispense agents as indicated in Section C.6.7.

C.6.1.2. The Contractor shall accept prescription orders at TMOP by written (original or facsimile), electronic (supporting digital signature including e-prescribing), or telephonic submission.

C.6.1.3. The Contractor shall notify the beneficiary when a prescription order is received directly from a prescriber and inform the beneficiary the order will attempt to be filled. Additionally, a method to communicate with the Contractor for questions will be provided to the beneficiary. The Contractor shall follow the electronic notes when submitted by the prescribers with an electronic prescription based on the current NCPDP standard.

C.6.1.4. The Contractor shall only accept prescriptions written by healthcare providers licensed in the U.S. with prescriptive authority as delegated by State and Federal law. A provider who meets the applicable standards of state or federal licensure, but who operates by the Indian Health Service, an Indian tribe, tribal organization, or urban Indian organization meets the requirements to have prescriptive authority under DHA rules.

C.6.1.5. The Contractor shall have procedures in place to reasonably assess the validity of prescription orders.

C.6.1.6. For all agents dispensed through TMOP, the Contractor's tracking and dispensing procedures shall comply with Federal and State law and all applicable State Boards of Pharmacy requirements.

C.6.1.7. The Contractor shall provide 24 hours a day, 7 days a week access to a pharmacist by phone for beneficiaries receiving prescription agents through the TMOP.

C.6.1.8. The Contractor shall not collect sales tax on prescriptions dispensed by the TMOP.

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C.6.1.9. The Contractor shall not dispense TMOP prescriptions to a beneficiary with OHI, unless one of the following exceptions applies:

- The OHI does not cover the prescribed agent, either by the benefit design or denied coverage review.
- The beneficiary has exhausted the benefits under the OHI.

C.6.1.9.1. Under these exceptions, to receive TRICARE coverage of agents dispensed through the TMOP, beneficiaries with OHI must submit documentation to the Contractor showing the OHI does not cover the prescribed item, or documentation such as an Explanation of Benefits (EOB) indicating their coverage has been exhausted. In cases where the Contractor is the PBM for the OHI, it may provide such documentation in place of the beneficiary. The Contractor will then update the beneficiary's profile with this information and process the prescription(s) accordingly.

C.6.1.10. The Contractor's TMOP prescription processing and written notification of denied orders for non-specialty agents shall meet the following minimum standards:

C.6.1.10.1. 98% of Mail Order prescriptions not requiring intervention or clarification shall be shipped within four (4) business days from receiving the prescription

C.6.1.10.2. 100% of Mail Order prescriptions shall be shipped, scheduled for delivery, returned, or denied within ten (10) calendar days of receipt, date stamped in accordance with TOM Chapter 8, Section 1.

C.6.1.10.3. Exceptions to the above prescription processing standard are as follows:

C.6.1.10.3.1 Prescriptions under the Deployment Prescription Program (DPP) that require clarifications or intervention. These will not be included in the calculation of mail order pharmacy processing time; and

C.6.1.10.3.2 Prescriptions for specialty agents which will follow delivery protocol specific to treatment needs per C.4.5.2.

C.6.1.10.3.3 Prescriptions dispensed from the TMOP shall be accurate 100% of the time, measured monthly.

C.6.1.11. In the event the Contractor fails to mail any prescription that did not require clarification or intervention within ten (10) business days for non-specialty agents or per beneficiary's need in the case of specialty agents, the Contractor shall automatically provide next day delivery service at no additional charge to the beneficiary.

C.6.1.12. The Contractor shall provide reporting of mail order volumes and processing times (CDRL Q040).

C.6.1.13. The Contractor shall contact the prescriber for each TMOP prescription received requiring intervention or clarification.

C.6.1.14. If the Contractor is unable to obtain a response from a prescriber within two (2) business days for a non-specialty agent or within an appropriate time necessary to dispense a specialty agent

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given a beneficiary's need, the Contractor shall contact the beneficiary based on the beneficiary's indicated preferences, to provide order status.

C.6.1.14.1. For interventions such as PA, MN or QL overrides, the Contractor shall request beneficiary direction to process the prescription within the given benefit design, postpone the prescription, or cancel the prescription, or continue efforts to obtain a response from the provider.

C.6.1.15. For prescription clarification, which requires the information on the prescription to be clarified for completeness, accuracy and legibility, the Contractor will request beneficiary direction to either postpone or cancel the prescription or continue efforts to obtain a response from the provider. The Contractor shall document all calls at the beneficiary's direction. The Contractor will be held to the 10-day processing requirement per C.6.1.9 for all non-specialty agents.

C.6.1.16. If an agent is unavailable at Mail Order, the Contractor shall notify the beneficiary, provide options where the prescription can be filled including in-network options, and complete the transfer of the prescription if requested. All notifications that do not require a written letter will be communicated through the beneficiary's preferred communication method.

C.6.1.17. The Contractor shall not postpone or cancel a prescription without first attempting to contact the beneficiary telephonically or by electronic means. The Contractor shall ensure the beneficiary has no less than 72 hours to provide a response before the prescription is postponed or cancelled.

C.6.1.18. For all returned or cancelled prescriptions, the Contractor must provide written notification to the beneficiary explaining why the prescription was returned or cancelled.

C.6.1.19. If the Contractor is unable to fill a prescription because the agent is on backorder or no longer available, the Contractor shall notify the beneficiary at the time of the order and offer to postpone the prescription, transfer the prescription to a retail pharmacy, postpone or cancel the prescription, or contact the provider to request an alternative agent which is available. If the beneficiary opts to transfer the prescription to a retail network pharmacy, it shall be processed in accordance with C.3.1.

C.6.1.19.1. For specialty agents that are unavailable at Mail Order, the Contractor shall follow the requirements in C.6.8.

C.6.1.20. If the prescription is postponed due to backorder, the Contractor shall contact the beneficiary when the agent is back in stock to request permission to fill the order.

C.6.1.20.1. The Contractor shall provide notification to the Government of items currently on backorder.

C.6.1.21. The Contractor will use industry best practices to maximize generic substitution, including attempts to convert Dispense as Written (DAW) prescriptions. TMOP prescriptions dispensed shall adhere to the Government's mandatory generic policy unless determined otherwise per benefit design. The Contractor shall support the use preferred biologic agents of per C.7.3.1.6

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C.6.1.22. Upon receipt of a DAW prescription for a brand name product for which a generic equivalent is available, the Contractor shall contact the prescriber to change the prescription to a generic equivalent or request an appropriate PA be submitted by the prescriber as described in C.8.

C.6.1.22.1. If the PA is denied or if the prescriber cannot be contacted, the Contractor shall cancel and/or postpone the prescription and notify the beneficiary per C.6.1.15.

C.6.1.23. For all denied mail order prescriptions, the Contractor shall provide notification to the beneficiary explaining why the order was denied and detailing the beneficiary's appeal rights.

C.6.1.24. At the direction of the COR, the Contractor may dispense brand in lieu of generic in instances where the brand is the lowest cost available on the medical pricing catalog, provided by the Defense Logistics Agency Troop Support (DLA-TS), for replenishment.

C.6.1.25. The Contractor may offer a refill reminder program but shall exclude specific agents from this service as determined by the DoW P&T Committee process. Specialty agents shall not be included in the refill program unless otherwise specified by the DoD P&T Committee.

C.6.1.25.1. The Contractor shall present beneficiaries a clear and easy way to opt-in and opt-out agents and allows them to make their selection on all electronic or web-based tools. Confirmation will be sent to the beneficiary upon selection via the beneficiary's preferred electronic means of communication.

C.6.1.25.2. The Contractor shall notify beneficiaries when a prescription enrolled in the refill reminder program is eligible for a refill. The Contractor shall only fill the prescription after a beneficiary provides confirmation that the prescription fill is needed.

C.6.1.25.3. The Contractor shall not dispense prescriptions exceeding 450 days' supply over the course of any 365-day period unless an override is deemed appropriate per C.8.10. This limitation will not be applied to beneficiaries during the period they are receiving DPP dispensing(s).

C.6.1.25.4. When a prescription expires, the Contractor shall contact the beneficiary to receive authorization to request a new prescription from the provider and to get confirmation from the beneficiary to continue enrollment of that prescription into the refill reminder program. Beneficiary confirmation that the prescription is still needed must be obtained by the Contractor prior to dispensing of the renewal prescription.

C.6.1.25.5. When an agent is removed from the refill reminder program, the Contractor shall notify the beneficiary per C.9.3.

C.6.1.25.6. The Contractor shall provide reporting in accordance with CDRL M130.

C.6.1.26. The Contractor shall use clinical best practices to ensure prescriptions are processed to reflect the beneficiaries' most recent therapy. This shall include but not limited to:

- Ensuring prescriptions processed have the most current strength, dosage forms, and directions of use.

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- Renewal prescriptions shall be retained in the beneficiary profile if too soon to be filled. The beneficiary will be notified of prescription receipt and anticipated processing date based on their preferred communication method. If the renewal prescription is enrolled in the refill reminder program, the beneficiary will be required to confirm if the prescription should be filled based on need.

C.6.1.27. The Contractor shall report dispensed ingredients that are replenished at the burdened unit price from the medical pricing catalog provided by the DLA-TS. On the TED, this will be populated as the Amount Paid by Government Contractor, per the TSM. Ingredients acquired by Contractor through MPPP should reflect the acquisition cost to the Government. See Section C.6.8.22 for more information.

C.6.1.28. The Contractor shall accommodate all special requirements in regards to handling, processing, or shipping of agents as recommended by the Food and Drug Administration (FDA) or the manufacturer of the product dispensed through TMOP.

C.6.2. Mailing Prescriptions

C.6.2.1. The Contractor shall mail TMOP prescription orders to only those beneficiaries living in the 50 United States, the District of Columbia, Puerto Rico, U.S. Virgin Islands, Northern Mariana Islands, American Samoa, and Guam, and to beneficiaries with an Army Post Office (APO), Fleet Post Office (FPO), or Diplomatic Post Office (DPO) address. The Contractor will follow best industry practices for delivering temperature sensitive products and will follow specialty handling requirements per C.6.1.26.

C.6.2.2. The Contractor shall mail TMOP prescriptions to an APO, FPO, and DPO address using, at a minimum, a delivery method equivalent to U.S. Priority Mail. All other TMOP prescriptions shall be shipped or mailed postage paid to the beneficiary in a manner which provides, at a minimum, a delivery time equivalent to First Class U.S. Mail. Prescriptions for specialty agents shall be mailed in accordance with delivery management services per C.4.5.3. as appropriate.

C.6.2.3. The Contractor shall ship TMOP prescriptions at no additional cost to the beneficiary. The Contractor may also offer pick-up at a local location, in accordance with state and federal laws.

C.6.2.4. The Contractor shall apply TMOP dispensing rules and cost-sharing amounts, regardless of delivery mechanism.

C.6.2.5. Upon request by the beneficiary, for non-specialty agents, the Contractor shall provide next day or other expedited delivery services to beneficiaries with a mailing address within the continental U.S. The Contractor shall ensure the beneficiary is responsible for the additional shipping cost at the Contractor's most favorable shipping rate.

C.6.2.6. The Contractor shall have the ability to suspend shipping to specified addresses outside the United States by postal code when directed by the Government.

C.6.2.7. The Contractor shall ship medications care of (c/o) to the beneficiary's health care provider's office, if requested by the beneficiary.

C.6.2.8. When shipping medications, the Contractor shall comply with U.S., U.S. Military, and U.S.

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Diplomatic Postal Service regulations.

C.6.2.9. The Contractor shall not ship prescriptions to addresses where any prior prescription or correspondence to that beneficiary has been returned to the Contractor as undeliverable without first contacting the beneficiary and verifying the address.

C.6.2.10. With each order shipped, the Contractor shall provide information on all options for reordering a medication.

C.6.2.11. The Contractor shall be responsible for all medications dispensed by the Contractor up to the verified point of delivery to the beneficiary or to the alternate delivery location designated by the beneficiary. For non-specialty agents when using U.S. Mail, the Contractor shall allow 12 days from the original ship date for the beneficiary to receive their order. Beginning 12 days after the ship date, the Contractor shall reship the order within three days of receiving notification from the beneficiary that their order has not been received or was received in an unusable condition. The Contractor shall ensure a beneficiary has up to 45 days from the original ship date to report that an order was not received and request a replacement. This shall be extended to 60 days for prescriptions sent using an APO, FPO, or DPO address. Specialty agents dispensed at TMOP shall follow the delivery model per C.4.5.2.

C.6.2.12. If the Contractor determines the original order for a non-specialty or specialty agent was not received or was received in an unusable condition, the Contractor shall reimburse the cost-sharing amount for the original order or may provide delivery of a replacement order with no additional cost-sharing amount. Upon receipt of beneficiary notification that the original order was not received, the Contractor shall educate the beneficiary regarding the cost-sharing amount for any replacement order and the potential for reinstatement of the original cost-sharing amount should the original order arrive in a usable condition. Following shipment of the replacement order, the Contractor shall monitor deliveries of prescription order(s) to determine the status of the original order as well as replacement orders and make outreach to the beneficiary as needed to address the ongoing issue.

C.6.2.13. If it is determined the original prescription order was not received or was received in an unusable condition, the Contractor shall not receive an administrative fee, replenishment or reimbursement for the lost or damaged prescriptions. The Contractor will receive an administrative fee, replenishment or reimbursement if the replacement is the result of a circumstance in which the Contractor has limited or no control, such as a agent recall or natural disaster. The Contractor shall report on all replacement shipments requested and fulfilled (CDRL Q041).

C.6.2.13.1. In situations where either a return is received or a request to return a prescription is received due to error(s) not caused by the Contractor or the beneficiary (see C.6.5), the Contractor shall work with the Government to determine appropriate resolution.

C.6.2.14. Order Tracking. The Contractor shall provide an electronic method for beneficiaries to monitor a prescription order from the day an order is processed to final delivery. Tracking should include updates on order processing, any processing delays, the date of expected delivery, and upon successful delivery. Additional notification would apply if an order is placed on the beneficiary's profile with a next eligible fill date, the order is eligible for release from postponing status and the order is postponed or cancelled. The Contractor shall ensure beneficiaries have an option to

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configure notifications if desired, based on their communication preferences.

C.6.2.15. The Contractor shall offer beneficiaries the option to schedule delivery dates for medications refilled (to include refill reminder program prescriptions and initial fills for renewed prescriptions) at TMOP.

C.6.2.15.1. When appropriate, the Contractor shall ensure beneficiaries are able to access scheduled delivery options online (website or mobile application), by phone or through their preferred communication channels.

C.6.2.15.2. Scheduled delivery shall align with the DoD benefit design.

C.6.2.15.3. Refills with a scheduled delivery date shall be retained in the beneficiary's profile until the appropriate time is reached to begin processing the prescription to meet the delivery date. Beneficiaries shall be able to postpone and change the delivery date for previously scheduled prescriptions prior to release. Once released, refills shall meet the minimum processing standards in accordance with C.6.1.9.

C.6.3. Mail Order Pharmacy Accounts

C.6.3.1. The Contractor shall support TMOP registration by a variety of means, including but not limited to submissions in writing, via telephone, or via the Contractor's website.

C.6.3.2. For prescription orders, the Contractor shall allow beneficiaries to provide a credit card for the cost-sharing amount.

C.6.3.3. The Contractor shall establish individual accounts for family members and shall allow for more than one credit card to be on record for collection purposes.

C.6.3.4. The Contractor shall ensure that if a beneficiary overpays a cost-sharing amount, the beneficiary is notified that the excess has been credited to the beneficiary's account for future prescriptions, or the overpayment is refunded to the beneficiary along with the explanation of the refund, based on the beneficiary's preference.

C.6.3.5. As a result of its own business judgment and at its own risk, the Contractor may choose to extend credit to beneficiaries so that when an insufficient cost-sharing amount is received, the Contractor may fulfill the prescription order up to the amount of the Contractor-established credit limit and credit aging parameters. As the Contractor is not acting as an agent of the Government in extending credit to beneficiaries, none of the recoupment procedures set forth in this contract or the TRICARE manuals shall be available to the Contractor to collect beneficiary cost-sharing amounts. Likewise, any uncollected debts from beneficiaries resulting from the extension of credit are not reimbursable under this contract. If the Contractor does not extend credit or the beneficiary has exceeded the Contractor's established credit parameters, the Contractor shall return the prescription to the beneficiary and notify the beneficiary of the correct cost-sharing amount required.

C.6.4. Returns

C.6.4.1. Prior to the start of pharmacy services, the Contractor shall identify its preferred Returns Management Reverse Distributor (Reverse Distributor) to the Government.

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C.6.4.2. The Contractor shall segregate all returned agents from all other agents in its facility.

C.6.4.3. The Contractor will further distinguish between returns for which replenishment of the agent is expected from the NPV per C.6.8.1, and any returns leading to a replacement furnished by the Contractor.

C.6.4.4. Any returns leading to a Contractor furnished replacement may be submitted to the Contractor's Reverse Distributor; any credits arising from these returned agents accrue to the Contractor.

C.6.4.5. For all agents returned to the MOP, the TED record will be adjusted or cancelled as necessary to properly reflect co-payment, administrative fee, and replenishment. The TED adjustment/cancellation must maintain an accurate clinical record on the PDW.

C.6.4.6. The Contractor shall hold all returned agents for which replenishment is expected for processing by the Government's Reverse Distributor.

C.6.4.7. The Contractor shall contact the Government's Reverse Distributor no less than quarterly to arrange for a return shipping date. The Contractor will provide the Government's Reverse Distributor access to its facility(ies) for onsite inventory, packaging, and shipment of returns to the Government's Reverse Distributor's central location.

C.6.4.8. The Contractor shall submit to the COR all receipts provided by the Government's Reverse Distributor upon pick-up. The Contractor is not responsible for the cost of packaging or shipment of returns to the Reverse Distributor.

C.6.4.9. The Contractor shall work directly with the Reverse Distributor to negotiate any disputes which may arise from the processing of returnable agents and/or the disposal of non- returnable agents owned by the Contractor. In the event, a dispute cannot be resolved, the Contractor may contact the COR and/or the CO for assistance.

C.6.5. Error Reporting

C.6.5.1. The Contractor shall provide a report on all defects and errors that occurred at TMOP (CDRL Q042). For purposes of this report, the Government defines a medical error according to the National Coordinating Council for Medication Error Reporting & Prevention (NCC-MERP): "Any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is in the control of the health care professional, patient or consumer." The Contractor shall classify medication errors according to the NCC-MERP medication error index.

C.6.5.2. All such events which are identified and corrected prior to leaving TMOP are considered defects.

C.6.5.3. If an error is discovered that impacts multiple stakeholders (e.g., benefit design errors, miscommunication, data integrity, claims processing, etc.) the Contractor shall provide a detailed correction plan which explains the root cause, the financial and beneficiary impact. This plan shall include set timelines for correction and a mitigation strategy to prevent further occurrences.

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C.6.6. Expanded Use of MTF and TMOP

C.6.6.1. The Contractor shall apply the requirements of the Expanded Use of MTF and TMOP (10 U.S.C. § 1074g(a)(9) as implemented by 32 CFR § 199.21(r)), in accordance with TPM, Chapter 8, Section 9.1, The Contractor shall report monthly results using the Expanded Use of MTF and TMOP Summary Report (CDRL M120).

C.6.6.2. The Contractor shall monitor and apply any changes to the list of select medications for Expanded Use of MTF and TMOP posted in accordance with TPM Chapter 8, Section 9.1.

C.6.6.3. The Contractor shall approve overrides to the Expanded Use of MTF and TMOP, which may be authorized for the situations listed below. The Contractor shall apply the waiver in the patient profile which will allow a universal override for future retail dispensing in accordance with TPM Chapter 8, Section 9.1.

C.6.6.4. The Contractor shall implement a waiver process based on personal need, hardship, emergency, or other special circumstances that requires use of a retail pharmacy, per TPM Chapter 8, Section 9.1 and criteria established by the DoD P&T Committee (February 2015). The determination shall be made using Contractor-developed, Government-reviewed criteria. Criteria shall include the follow circumstances:

- A beneficiary exhibits personal need, hardship or an emergency.
- A beneficiary residing in a nursing home or other long term care facility. Communication with the beneficiary, a relative or a caregiver, is sufficient to establish residency in a nursing home. The Contractor will allow caregivers to establish residency for multiple beneficiaries at the same time.
- Barriers exist preventing a beneficiary from receiving medications by mail (e.g., no permanent mailing address, resides in rural setting).
- Special circumstances shall include but are not limited to the following:
 - The dosing is not stable or the agent requires titration to achieve therapeutic effectiveness or safety.
 - Prescriptions for different specialty agents for a given beneficiary are split between multiple pharmacies in the network due to limited distribution.
 - Medically fragile beneficiaries (e.g. pediatric, complex and serious medical conditions, disability due to health impairment, etc.) receiving specialized care and services through a retail network pharmacy.
 - Clinical and/or situational needs that would be better served through a specific retail network pharmacy.

C.6.6.5. The Contractor shall provide a dedicated toll-free number to assist beneficiaries in transferring prescriptions from various POS based on the beneficiary's preference.

C.6.6.6. When the Contractor processes a retail network pharmacy claim for a beneficiary subject to Expanded Use of MTF and TMOP, the Contractor will communicate information regarding the options available to the beneficiary.

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C.6.6.7. The Contractor shall contact the beneficiary by letter or electronic means, based on the beneficiary's preference, by the end of the following week after each of the two potential 30 calendar day courtesy fills and after the beneficiary exhausts the courtesy fills and has paid the full cost for their select maintenance medication at a retail pharmacy. The communication shall remind the beneficiary of their options for obtaining future fills (at MTF, TMOP or pay the full cost of the medication at a retail network pharmacy) and provide contact information for the Contractor's call center.

C.6.6.8. After the two 30 calendar day courtesy fills, the Contractor shall require beneficiaries to pay the full cost of prescriptions for select maintenance medications when dispensed at a retail network pharmacy unless the beneficiary meets requirements for waiver or override. When a beneficiary opts to pay full price for a select medication at a retail network pharmacy, it is considered a non-covered service. A record of the dispensing shall be posted to the patient's profile. The Contractor will not reimburse paper claims submitted by beneficiaries who paid the full cost of a select medication, unless otherwise authorized by the COR subsequent to a review.

C.6.6.9. The Contractor shall not reject prescriptions filled at VA pharmacies under this program.

C.6.7. Deployment Prescription Program (DPP)

C.6.7.1. The Contractor shall manage all aspects of the TMOP registration and prescription process for deploying beneficiaries and for beneficiaries at in-theater locations.

C.6.7.2. The Contractor shall provide comprehensive reporting, allowing the Government to monitor the program (CDRL M100). Deployment prescriptions include all prescriptions for TRICARE-eligible beneficiaries deployed in a theater of operation.

C.6.7.3. For beneficiaries deployed in theaters of operation, the Contractor shall provide an electronic method for beneficiaries to monitor a prescription order per C.6.2.13.

C.6.7.4. For beneficiaries deployed in theaters of operation, the Contractor shall provide prescription status notifications by electronic means or by telephone, based on the beneficiary's preference, including but not limited to prescriptions received, placed in postponed status, next eligible fill date, eligible for release from postponed status, prescription order shipment, as well as any prescription delays, returns or cancellations.

C.6.7.5. The Contractor shall receive DPP prescriptions via standard mail and electronic methods (such as fax, secure server, or e-prescribing system) from any Pre-Deployment or Out-Processing Center, hereafter referred to as the "Center(s)." DPP prescriptions will also be received using these same channels from within theater.

C.6.7.6. Upon receipt of a DPP prescription, the Contractor shall verify, adjudicate, and process the prescription in accordance with the procedures outlined in C.6.7.11. DPP prescriptions may have additional restrictions (e.g., psychotropic policy, non-deployable medications, etc.), as stipulated at the Government's Deployment Prescription Program page (Attachment J-3, Line 2). Specific restrictions may vary depending on which Combatant Command (e.g., CENTCOM, SOUTHCOM, PACOM, etc.) the deployment falls under. These restrictions are published by each Combatant Command.

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C.6.7.7. Specialty agents as defined by C.4 may be supported through the deployment program if the specialty agent is deemed eligible to be dispensed based on the restrictions per C.6.7.6 or if the beneficiary is waived from the restrictions and the specialty agent is considered suitable for shipment.

C.6.7.8. The contractor shall ensure the implementation of the most up to date benefit restrictions as they pertain to the Combatant Commands per C.6.7.6. The contractor will provide any updates to the benefit restrictions prior to implementation for Government review and approval.

C.6.7.9. The Contractor shall develop and maintain a prescription form to be used solely for the DPP, based on a template furnished by the Government. The Contractor's initial DPP prescription form, as well as any subsequent updates or modifications, must receive Government approval prior to dissemination and utilization. The Contractor's will ensure its DPP prescription form is able to be printed and completed by hand, as well as completed electronically with the ability to be digitally signed.

C.6.7.10. To support DPP prescriptions, the Contractor shall establish and maintain communications channels for all Centers and regional theater pharmacists.

C.6.7.11. The Contractor shall educate the Centers and regional theater providers, eligible deploying personnel, and MTF personnel on the DPP to include the most common causes for delays in prescription processing and returned prescriptions and shall actively work with the Centers and theater personnel to maximize the volume of prescriptions received and dispensed that do not require intervention or clarification.

C.6.7.12. The Contractor will provide a dedicated team to interface with deployment centers and theater personnel and establish channels of communication to align processes to submit/process prescriptions.

C.6.7.13. Upon receipt of a DPP prescription, the Contractor shall perform the following verification steps:

C.6.7.13.1. The Contractor shall perform a DEERS eligibility query. If a DEERS query shows the beneficiary as ineligible for MOP, the Contractor shall notify the beneficiary with explanation, and provide guidance so the beneficiary may take the appropriate steps needed to show eligibility. Activated National Guard and Reserve service members may need to submit orders to DEERS to update their active-duty status.

C.6.7.13.2. The Contractor shall verify that registration is complete and legible. The Contractor shall return the prescription form to the Centers for correction as necessary. For theater prescriptions, the Contractor shall contact the beneficiary, or the provider if unable to contact beneficiary, to verify registration information.

C.6.7.13.3. The Contractor shall enter all beneficiary registration and prescription information into the Contractor's system and verify the information provided is complete and correct. If clinical issues are identified with prescription information, the Contractor shall coordinate with the Center or theater prescriber as appropriate, to resolve issues.

C.6.7.13.4. The Contractor shall verify that all prescriber identifiers are received and valid.

C.6.7.13.5. The Contractor shall screen prescriptions to be postponed for completeness and

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perform ProDUR prior to postponing. The Contractor shall ensure rejections and clinical warnings that would prevent the prescription from processing when released from postponed status are resolved with the Center or theater prescriber and corrected in the system prior to postponing the prescription. This screening is in addition to the full adjudication process that occurs prior to dispensing, when the prescription is released from postponed status by the beneficiary.

C.6.7.14. The Contractor shall notify the beneficiary via electronic means or telephone, based on the beneficiary's preference, when a postponed prescription is eligible to be filled.

C.6.7.15. The Contractor shall dispense prescriptions following the standard mail order pharmacy adjudication process. DPP prescriptions, to include specialty medications, will be prioritized for dispensing when a medication is of limited supply or subsequently becomes available from backorder status from the NPV. The Contractor will notify the beneficiary when the prescription can be filled per C.6.7.16.5.

C.6.7.16. For all DPP prescriptions, the appropriate override will be entered by Contractor to allow the prescription to fill. These overrides may include the following:

- Max days' supply limit (up to 180 days' supply)
- Refill too soon
- MN
- PA required
- Excluded Agent if available through TMOP (e.g. OTC or other agents deemed necessary for deployment.)
- QL
- Maximum Cost

C.6.7.17. Where clarification is required to process the prescription, the Contractor shall contact the prescriber (if originating from theater) or Center (if originating from Pre-Deployment Center) within one (1) business day to verify the prescription, and to notify the beneficiary of the issue.

C.6.7.17.1. The Contractor shall make a second attempt at 7 calendar days after initial outreach if no response has been received. The Contractor shall allow up to 14 calendar days for a response.

C.6.7.17.2. For prescriptions originating in-theater, the Contractor will utilize the regional point of contacts (POCs) to assist in resolution. After attempting appropriate follow-up, the Contractor shall postpone and/or cancel any prescriptions that cannot be processed to the appropriate POC and notify the beneficiary and the prescriber.

C.6.7.17.3. When a delay is expected, the prescription cannot be processed, and upon successfully processing and/or shipping, the Contractor shall notify the beneficiary via electronic means or by phone based on the beneficiary's preference, within one (1) business day.

C.6.7.17.4. The Contractor shall prioritize DPP prescriptions for dispensing, to include specialty medications, that have been postponed due to limited supply when a medication subsequently becomes available from backorder status from the NPV. The Contractor will notify the beneficiary once the prescription can be filled per C.6.7.14.1

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C.6.8. Agents and Supplies

C.6.8.1. The Contractor shall provide inventory of agents for TMOP dispensing to TRICARE beneficiaries. The Contractor shall be responsible for ordering all agents (specialty and non-specialty). The Government will replenish that inventory as set forth below.

C.6.8.1.1. The Contractor shall use the NPV as the primary source for replenishing agents and supplies unless the Government approves dispensing of specific agents from commercial sources under the MPPP (see C.6.8.22). The Contractor shall request replenishment by using the medical pricing catalog to obtain agents from the Government's contracted NPV.

C.6.8.2. For the Government to replenish agents dispensed to TRICARE beneficiaries, the Contractor shall request replenishment for the same agents (agent, strength, and form) as dispensed.

C.6.8.3. To optimize replenishment, the Contractor shall provide written notification to the COR when the NPV is not able to resolve situations which may impede the replenishment process. This notification will occur via CDRL D010 or R130.

C.6.8.4. The Government, through the DLA-TS, compiles the medical pricing file from Federal Supply Schedules (FSS), Distribution and Pricing Agreements (DAPA), joint DoD/DVA national contracts, DoD contracts, and Blanket Purchase Agreements (BPA). The Contractor shall use the medical pricing file to identify and select the most economical agents to the Government which can support utilization volumes for replenishment. The medical pricing file will also be used to price claims dispensed from TMOP. Orders shall be rounded down to the nearest whole package size for the product needed for replenishment. The agents will be shipped by the NPV to the Contractor's TMOP location(s). When required by the NPV or manufacturer, the Contractor may receive orders that are drop shipped directly from the manufacturer to the aforementioned locations.

C.6.8.5. The Contractor shall not deviate from the procedures described above when ordering products from the NPV without prior written authorization from the COR or KO. Requests to do so shall follow C.6.8.17.

C.6.8.5.1. The Contractor shall submit electronic orders to the NPV 24 hour, 7 days a week. Routine manual orders shall be submitted Monday through Friday (normal business days) between the hours of 8:00 a.m. and 5:00 p.m. Orders must be received prior to 5:00 p.m. local distribution center time to be considered for next business day delivery. Orders received after 5:00 p.m. will be shipped two business days after the date of the order.

C.6.8.5.2. The Contractor shall be limited to two emergency shipments per month for each of the TMOP facilities at no additional transportation/handling charges to the ordering facility. Any additional emergency shipments requested by the ordering facility shall be at the expense of the ordering facility which shall include all applicable transportation and handling costs.

C.6.8.6. The Contractor shall track and report volume of dispensed agents and the replenishment agents ordered and received from the NPV. The Contractor shall also track and report the volume of agents dispensed from approved MPPP (see C.6.8.22) recommendations and provide auditable reconciliation reporting by NDC number (CDRLs M110, M111, M140 and R140). The elements

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required for auditing will be specified by the Government. In the event that a dispensed NDC is not available for replenishment from the NPV, the Contractor will request replenishment with a therapeutically equivalent substitute product with the same agent, dose, and dosage form. In cases where therapeutic equivalent agents are used to replenish dispensed agents, the Contractor will maintain records which permit the therapeutic equivalent agents to be tracked by quantity and cost back to the original agents in the Contractor's inventory. The quantities ordered from the NPV for replenishment shall not exceed the quantities dispensed.

C.6.8.7. The Contractor shall reconcile any latent replenishment owed, no more than 90 calendar days after conclusion of each option period. If the quantity owed cannot be depleted within this timeframe, the Contract shall notify the COR and/or KO to request an extension and provide a plan to reconcile within a reasonable amount of time (CDRL A140).

C.6.8.8. The Contractor will provide written notification to the Government within 10 calendar days if availability issues result in prescriptions being returned to the beneficiary.

C.6.8.9. The Contractor shall coordinate with the NPV and the DLA-TS as necessary to accomplish the replenishment of agents. The operational processes for this coordination are between the Contractor and the NPV but shall be consistent with NPV order monitoring requirements and the requirements established in the DLA-TS/NPV contract SPE2DX-20-R-0001, or successor contracts.

C.6.8.9.1. The maximum number of discrete contractor facilities at which the NPV can support replenishment is 12.

C.6.8.10. The Contractor shall work collaboratively with the NPV to achieve efficiencies with regards to communications, ordering and delivery practices, usage forecasting and necessary reporting to facilitate replenishment. At DHA's request, the Contractor shall coordinate and/or host meetings quarterly for these purposes which at a minimum will include DHA, DLA-TS, and the NPV.

C.6.8.11. The Contractor shall provide usage data to the NPV for prescriptions dispensed by TMOP. The data should allow the NPV to accurately forecast the quantities necessary to meet current usage demands. The Contractor shall notify the NPV at least 14 days prior to any situations which may impact usage demands (e.g., the movement of prescription volume between dispensing facilities, benefit design changes or any other situations that may impact usage demands) to ensure the NPV is able to support such changes.

C.6.8.11.1. For newly marketed items of supply, the Contractor shall notify the NPV at least 30 calendar days before changes in its usage patterns. Newly marketed items of supply are items approved by the FDA within the past six months.

C.6.8.11.2. The Contractor shall notify the NPV if any item is being removed from its usage data. The data will be removed by the NPV effective the last day of that calendar month.

C.6.8.12. The Contractor shall participate in a process to expend any credits that have accumulated with the NPV as a result of returns, pricing errors, and other adjustments. In the event of an unusually large credit, the Government may initiate an out-of-cycle request to perform the process.

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C.6.8.13. As provided herein, the Contractor shall primarily dispense NPV replenishable agents. The Contractor understands that dispensed agents that are non-replenishable from the NPV are the responsibility of the Contractor. When an agent that is normally available from the NPV becomes unavailable, the Contractor will utilize its inventory to fill prescription orders, until NPV replenishment occurs, or a MPPP recommendation if approved by the Government under C.6.8.22.

C.6.8.14. A dispensed agent that has not been replenished is deemed non-replenishable (subject to Government review and concurrence) in the following cases:

- If the agent is a discontinued brand product which has been removed from the market and cannot be replenished. This does not include cases in which a branded agent has changed to a new manufacturer and the branded agent becomes available with a different NDC.
- If the agent is a discontinued generic product, for which a therapeutic equivalent generic product cannot be replenished.
- Where the Government has directed a change from a brand product to a generic equivalent associated with a market shift to the generic product.
- Agents which have limited use (subject to Government review and concurrence) but are necessary to support troop readiness or to maximize replenishment (e.g., DAW prescriptions indicating a specific manufacturer, on demand agents with limited demand).
- Agents which have no commercial use (subject to Government review and concurrence) but are necessary to support troop readiness or to maximize replenishment (e.g., agents with no commercial use but are used for troop readiness, agents made available to the Government by manufacturers which contractor does not have a commercial relationship with).
- Situations in which the Contracting Officer or the COR determine that replenishment is not appropriate.

C.6.8.15. Annual Re-baseline. Prior to the start of pharmacy services and by a date mutually agreed upon after award, and during each successive exercised option period within 60 days of CO notification, the Contractor shall submit a baseline listing of multi-source generic or branded products by NDC for approval by the CO or COR (CDRLs W010 and W011). Each baseline listing will identify the therapeutically equivalent NDC that is the lowest cost agent available to the Government and will be dispensed for each product whenever substitution is permitted by the prescriber.

C.6.8.16. Continuous Monitoring. Following the start of option period 1, the Contractor shall continuously monitor availability and pricing of all replenished products and provide weekly recommendations to the Government for the lowest cost agents available to the Government to be dispensed (CDRL W010 and W011). The Contractor shall identify the recommended change by NDC for the Government's approval and the anticipated annual savings to the Government based on current utilization trends. The Contractor shall exclude those products acquired under a commercial market recommendation approved by the Government per C.6.8.22.1, during the period in which the Contractor is dispensing the commercial products.

C.6.8.17. Authorization for NDC change requests:

C.6.8.17.1. NDC change request for agents replenished must be sent to the CO or COR for review and approval (CDRL D010 and R130). The Government may direct the Contractor to make additional changes due to: 1) significant changes in agent prices, 2) the Government's award of a

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agent procurement contract, or 3) other circumstances that require a change.

C.6.8.17.2. In the event, an NDC change takes longer than 30 calendar days to complete, the Contractor shall notify the Government per C.6.8.18

C.6.8.17.3. Agents selected for Government approval must align with the benefit design (e.g. Brand over Generic, 703b restrictions).

C.6.8.17.4. The Contractor shall complete each NDC change no later than 30 calendar days after being notified by the Government. The Contractor shall submit a written request for extension to the COR within 10 days of receiving initial notification if the NDC change is expected to take longer than 30 calendar days. The request shall state the date the NDC change will be made and include the rationale for the extension.

C.6.8.17.5. In situations where the NPV has supplies of government-specific inventory, the Contractor shall work with the NPV to deplete existing supplies prior to implementing an NDC change or dispensing the commercially acquired pharmaceutical agents. The Contractor shall notify the Government if it is unable to deplete this inventory.

C.6.8.17.6. Standard Receipt and Acknowledgement Process. The ordering facility shall conduct a physical inventory and transmit an accurate EDI 527 Material Due-In and Receipt transaction to acknowledge receipt of all delivered orders. This transmission must occur within 48 business hours of the delivery confirmation time. In the event the NPV's agent does not obtain a signature, the delivery confirmation time shall be the timestamp recorded by the NPV's delivery tracking system upon arrival at the ordering facility's geographic address, provided such timestamp is electronically verifiable. The ordering facility's signature on the bill of lading constitutes acknowledgement of the receipt of a specific number of sealed pallets or totes only. It does not constitute acceptance of the contents therein until the physical inventory is complete. Any evidence of tampered seals or damaged totes must be noted on the bill of landing at the time of delivery to be considered valid.

C.6.8.17.7. Prohibition of premature or inaccurate submissions. Submitting an EDI 527 transaction reflecting a zero or partial receipt for any order that has not been physically inventoried and confirmed by the ordering facility is a direct violation of this requirement. Such submissions will be considered invalid.

C.6.8.17.8. Consequences for non-acknowledgement. If the ordering facility fails to transmit the EDI 527 transaction within the 48 business hours window, the order shall be deemed received in full. In such cases, the ordering facility waives all rights to report any subsequent discrepancies including shortages, overages, or damaged goods, for that specific order.

C.6.8.17.9. Process for Exception due to Force Majeure. If a Force Majeure event (e.g., national disaster, facility lockdown) prevents the ordering facility from complying with the 48 business hour requirement, the designated facility manager shall provide written notification to the COR within 24 hours of the event's onset. This notification shall include a brief description of the event and an estimated timeline for completing inventory. A reasonable extension to the deadline may be granted at the sole discretion of the COR. Staffing shortages, equipment failures, or other routine operational challenges shall not constitute a Force Majeure event.

C.6.8.17.10. Discrepant Orders. The ordering facility(s) shall report all discrepancies relating to

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an order delivered by the NPV within 48 hours after delivery. Discrepancies include but are not limited to the following conditions:

- Products were shipped in error.
- Overages or short ships.
- Products with concealed damages or damaged in shipment.
- Products that did not meet the expiration/shelf life dating requirements of the NPV contract, unless otherwise authorized by the ordering facility or the DLA Troop Support NPV Contracting Officer.
- Products that were recalled, regardless of level of recall, except when the manufacturer's policy states otherwise, in which case the manufacturer's disposition instructions shall be followed.
- Other conditions consistent with the NPV's normal return policy.

C.6.8.17.11. TMOP facility(ies) shall report all discrepancies to the NPV customer service center within 48 hours after receiving an order. Discrepancies shall be communicated in a manner mutually agreed upon by all parties to include interactive work platforms. DLA Troop Support Discrepancy Mailbox (Attachment J-3, Line 15) shall be included on any communication to the NPV regarding discrepancies. Any discrepancies reported after 48 hours may be rejected by the NPV. In limited instances, a discrepancy may require additional research to reach a final conclusion which shall have a mutually agreed upon date between the NPV and the ordering facility. Notification of these instances and the agreed upon resolution date shall be reported to the NPV, DLA and DHA as soon as they are identified.

C.6.8.17.12. If an ordering facility receives an overage, or agent is received after the NPV cancelled the item on the order confirmation, the ordering facility shall notify the NPV and obtain disposition for the items. The NPV is to either provide a return authorization number and arrange for pickup or inform the ordering facility of other actions. The ordering facility is not authorized to keep the overage or cancelled agent. Discrepant goods will be held by the ordering facility's receiving point subject to NPV's disposition instructions.

C.6.8.17.13. The Contractor shall submit all discrepancies regarding cold chain products directly to DLA Troop Support Cold Chain Management, where it will be evaluated for product quality. When valid discrepancies are identified, DLA Troop Support Cold Chain Management will notify the NPV of the specific error and indicate when receipt for goods should not be posted by the Contractor or if a credit must be given.

C.6.8.18. Market Priced Pharmaceutical Program (MPPP)

C.6.8.18.1. The Contractor may recommend to the Government sustainment offers of Trade Agreement Act (TAA) compliant agent(s) to dispense at TMOP or specialty pharmacies where supply sources are not stable or are unavailable through the NPV(s). Such offers may address situations when: (1) the only alternative agents available from the NPV(s) are branded agents, or agents otherwise treated as brand, which are more costly to the Government compared to generic agents available in the commercial market; or (2) when there is accumulation of replenishment owed. The Government may impose additional restrictions such as minimum order thresholds (e.g., offers with an estimated duration less than 30 days).

C.6.8.18.2. The Generic Sequence Number (GSN) shall be the means of identifying the recommended agent family.

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C.6.8.18.3. The Contractor's MPPP recommendation shall provide the following:

- The specific medical pricing catalog NDC and price for which a therapeutically equivalent agent is being offered.
- Evidence of market conditions which support the Contractor's recommendation based on the Government's usage demands at the time of evaluation.
- An attestation that the product is therapeutically equivalent to the comparable agent listed on the medical pricing catalog.
- An attestation that the agent is TAA compliant.
- The unit price and quantity available from the recommended commercial source.
- The proposed total cost to include agent, administrative fee, and estimated incentive.
- The expected allocation of agent, utilization between DoD and Dual-Eligible Beneficiaries.
- Cost savings of the combined agent and administrative fee when compared to the listed price on the MMC.
- The expiration date of the offer, as well as the estimated time period during which the product shall be dispensed based on current usage demands.

C.6.8.18.4. The Contractor shall inform DLA concurrently of offers it submits to the CO. The information provided to DLA shall include the NDC but shall exclude contractor pricing information.

C.6.8.18.5. If the Contractor's MPPP recommendation is approved by the Government, a provisional approval will be communicated to the Contractor, who will then communicate the specific NDC to be purchased to the Government. The Government will then use the NDC to make an independent determination of therapeutic equivalence and TAA compliance.

C.6.8.18.6. Upon the Government's confirmation of the Contractor's attestations, the parties will execute a modification identifying the agent, unit price, and quantity at which the Contractor is authorized to purchase and dispense the specified agents (M140 and R140) in lieu of ordering replenishment. The Contractor will be paid an administrative fee for each recommendation which is accepted by the Government and implemented and may request reimbursement for the cost of agents (described herein) dispensed by invoicing as described in Section G.

C.6.8.18.7. If the NPV has excess quantities of government-specific inventory on hand (for the purpose of replenishment), the Contractor shall coordinate with the NPV to deplete these supplies per C.6.8.19 prior to dispensing the commercially-acquired agents through the MPPP. All agents acquired through the MPPP shall be dispensed before dispensing the products obtained through replenishment. The Contractor shall notify the NPV, to ensure product is available to support the Government usage demands, prior to when it has completed the dispensing MPPP-acquired agents and allowing for seamless transition back to use of Government replenished inventory.

C.6.8.18.8. At the request of the Government, the Contractor may submit MPPP offers to meet the requirements of MTF pharmacies. Any additional terms and conditions needed to facilitate this support may be negotiated as needed.

C.7. Formulary and Benefit Design

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C.7.1. DoD Uniform Formulary

C.7.1.1. The Contractor shall comply with the provisions of the DoD Uniform Formulary and its cost-sharing amount structure. Uniform Formulary changes are generally announced quarterly in the DoD P&T Committee Minutes (Attachment J-3, Line 1) and on the TRICARE website (Attachment J-3, Line 14).

C.7.1.2. The Contractor shall participate in the formulary review process as a voting member of the Beneficiary Advisory Panel (BAP). Further information is available at the BAP website (Attachment J-3, Line 5).

When the P&T process makes changes to the formulary, (such as the approval of new or revised clinical restrictions) prior authorization, medical necessity, or updates clinical and/or safety criteria) the Contractor shall accurately adopt those changes according to the specified implementation date. The Contractor shall continually monitor and implement published uniform formulary changes (see Attachment J-3, Line 1).

C.7.1.3. The Contractor shall participate in quarterly meetings with the Government to align on implementation of benefit design changes. Meeting will be in-person or virtual as determined by the Government.

C.7.1.4. Cost-sharing amounts based on formulary status, point of service, and beneficiary category shall be charged to beneficiaries in accordance with the TRM, Chapter 2, Addendum B.

C.7.1.5. The Contractor shall provide a report (CDRL M200) documenting the implementation of benefit design changes and the identification and correction of inaccuracies.

C.7.2. Contractor Formulary Support

C.7.2.1. The Contractor shall augment and support the Government's efforts in developing methodologies for benefit design analysis and agent class reviews utilizing clinically appropriate best practices.

C.7.2.2. The Contractor's recommendations shall be advisory, evidence-based and clinical in nature, and include documentation and supporting materials to inform the DHA decision-making process. The scope of the recommendations shall include:

- outpatient formulary (UF/BCF/ECF) agents therapeutic class reviews
- recommendations for formulary status (e.g., UF, NF, step preferred, non- preferred, non-covered)
- prior authorization and medical necessity criteria
- quantity limits
- ingredients for compounds
- agents appropriate to include on the specialty agent list (C.4.1)
- inpatient agent formulary recommendations to support DHA efforts to standardize MTF inpatient and clinic administered agent utilization and dispensing.

C.7.2.3. The Contractor's formulary recommendations must be in compliance with 32 CFR 199.21, align with DoD benefit design rules, account for Congressionally mandated changes and DHA program needs, and meet cost-effectiveness objectives of the DHA while preserving access and

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readiness.

C.7.2.4. The Contractor shall monitor and provide timely notification to the Government of new agents that are scheduled to come to market and generally covered. The Contractor shall facilitate in identifying these agents as an innovator or a line extension and provide commercial formulary management strategies as available. In addition, the Contractor shall provide approval pathways (e.g., NDA, ANDA and BLA) and identify if new agents are authorized generics and biosimilar products all in effort to support formulary decision making.

C.7.2.5. The Contractor shall provide market agent trend reports, new agent pipeline analysis, and agents approved by the FDA with a BLA no less than quarterly (CDRL Q170).

C.7.3. Benefit Design

The Contractor shall ensure all changes to the benefit design are accurately implemented and maintained in accordance with the Government's intent. The Contractor shall obtain clarification if the Government intent is unclear prior to assigning benefit design changes (e.g. formulary status, copay structure, PA/MN criteria, etc.)

C.7.4 If the Contractor is not able implement the benefit design changes as stated in the DoD P&T minutes due a situation to a circumstance beyond the Contractor control, the Contractor shall request a waiver for review and consideration by the KO with an explanation as why the implementation was missed and timeframe for when the implementation will occur.

C.7.3.1. Use of Generics and Biologic Agents

C.7.3.1.1. The Contractor shall adjudicate prescriptions in accordance with the TRICARE pharmacy program's mandatory generic substitution policy under 32 CFR 199.21. The Contractor shall accept certain Dispense as Written (DAW) codes in accordance with state law or regulation. In limited situations, certain codes may be accepted as directed by the Government.

C.7.3.1.2. When directed by the Government, the Contractor shall regard designated brand medications as generics and designated generics as brand, applying the appropriate formulary status and copay. The P&T Committee decisions will govern the formulary status of a particular agent. This status may be designated for a specific point of service. The Contractor shall notify the Government when a brand / generic designation is changed (e.g Authorized Generic) as a commercial practice.

C.7.3.1.3. The Contractor shall notify the Government when a brand/generic designation is changed (e.g. Authorized Generic) as a commercial practice.

C.7.3.1.4. At the direction of the Government, the Contractor shall advise prescribers and/or beneficiaries of those biologic agents preferred by the DoD benefit design. This includes use of auto-substitutions when clinically appropriate, and prescriber and/or beneficiary out-reach to support substitution to the preferred agents. The Contractor shall provide recommendations to the Government of commercial processes and if these processes would benefit in promoting the preferred agents.

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C.7.3.2. New to Market Agents

C.7.3.2.1. The Contractor shall have up to 5 business days from the time a new agent comes to market (here by known as a new to market agent) and is available to adjudicate within the Contractor's system, to accurately implement benefit design rules as determined by the DoD P&T Committee or statutes that may apply to the new to market agent. The Contractor shall notify the Government in writing when the agent becomes available.

C.7.3.2.2. Any pharmacy claims for TRICARE beneficiaries for a new to market agent that occur prior to the accurate implementation of the benefit design will be the responsibility of the Contractor to correct and if necessary, make the beneficiary and/or the Government whole.

C.7.3.2.3. The Contractor shall provide the Government details of the impacted claim(s) that have adjudicated prior to the implementation of the benefit design and recommendations to address these claims. In situations where the Contractor believes the best course of action is for the beneficiary to continue therapy with the new to market agent, the Contractor shall provide the Government clinical rationale and request approval.

C.7.4. Vaccines

C.7.4.1. TRICARE authorized vaccines may be administered by retail network pharmacies in accordance with the Centers for Disease Control (CDC) immunization protocols (see Attachment J- 3, Line 6) and are covered under the TRICARE Pharmacy Benefit in accordance with TPM Chapter 7, Section 2.2. The Contractor shall maintain a list of covered vaccines and adhere to any future change to CDC immunization protocols.

C.7.4.2. The Contractor shall update vaccine availability immediately upon the new vaccine effective date, in accordance with TPM Chapter 7, Section 2.2. The Contractor shall make a list of covered vaccines publicly available for TRICARE beneficiaries.

C.7.4.3. The Contractor shall ensure that retail pharmacies adjudicate vaccine claims utilizing the most current vaccine NDC(s) available for those vaccines that are reformulated frequently (e.g. influenza vaccine, COVID-19). Prior vaccine NDC(s) should not adjudicate.

C.7.5. Infused and Injectable Medications. The Contractor shall process claims for infused and injectable pharmaceutical agents in accordance with the TPM, Chapter 8, Section 9.1 and Section 20.1.

C.7.6. Non-Covered Items. The Contractor shall deny requests for agents and supplies not covered under the pharmacy benefit in accordance with the benefit design. Appeals of denied claims submitted by the beneficiary will be reviewed under the appeals process, set forth in TOM, Chapter 12. Agents completely excluded from the benefit by the Director, DHA (per 32 CFR 199.21(e)(3)(ii)) will not be considered for an appeals review.

C.7.7. Compound Prescription Program. The Contractor shall manage a compound prescription program. The program shall, at a minimum:

- Align with industry best practices
- Ensure each ingredient in a compound follows DoD benefit design in real time.

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- Ensure evidence-based safety, clinical effectiveness and cost effectiveness
- Monitor and advise on opportunities to control compound spending and average cost per prescription
- Perform continuous surveillance to ensure appropriate compound billing practices
- Apply to compound prescription claims from MTFs processed under the Federal EHR

C.7.7.1. The Contractor shall provide a monthly report for compounded agents per CDRL M210.

C.7.7.2. Before dispensing any compounded medication through TMOP, the Contractor shall verify that all supplies and ingredients required to prepare the compound are available for replenishment from the NPV, as described in C.6.8. In the event that any required products are not available, the Contractor may return the prescription to the beneficiary.

C.7.7.3. Safety and Enhanced Care Services. The Contractor shall provide services to manage appropriate use of agents and treatment regimens for non-specialty agents to achieve measurable outcomes and cost controls. Management strategies shall engage beneficiaries, prescribers and/or pharmacies and incorporate evolving industry best practices or commercial standards. Outcomes shall include but are not limited to:

- Preventing adverse agent events
- Maximize use of preferred formulary agents which includes a managed approach to maximize use of preferred biologic agents
- Maximizing generic utilization
- Improved adherence
- Improved understanding of the DoD Benefit Design by beneficiaries, prescribers and/or pharmacies

C.7.7.4. The Contractor shall monitor potential non-adherence that might result from designated DoD benefit design edits (i.e., Prior Authorization, step therapy and non-covered), leading to prescription claim rejections or barriers that might prevent access to certain agents. The contractor shall conduct interventions to maximize continuity of care. This is independent of standard DUR safety edits and will only be implemented for Government designated agents and DoD benefit design edits and shall be suppressed for MTF upon request by the Government.

C.7.7.5. The Contractor shall coordinate with prescribers to maximize appropriate treatment regimens utilizing formulary preferred agents. The approach should include a benefit wide examination of where potential cost-savings opportunities exist and focus on those opportunities that yield the greatest savings to the Government. These opportunities will be provided in a quarterly report of the Government per C.7.8.3. agent

C.7.7.6. The Contractor shall provide a report, submitted no less than quarterly; (CDRL Q070):

- Actions taken to support continuity of care;
- Enhanced care outcomes; and
- Cost savings/avoidance.
- Opportunities to improve provider formulary compliance, recommended goals to be reached, documentation of contractor's effort to maximize provider utilization of preferred formulary items, how the contractor's effort is tracked and documented to determine if there is an improvement in formulary compliance from targeted providers.

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C.7.7.7. The Contractor shall provide a program to manage the use of opioid agents. The outcomes of this program shall be reported in CDRL Q210

C.8. Clinical Reviews and Overrides

C.8.1. The Contractor shall perform clinical reviews for agents designated by the P&T as requiring PA and/or MN and based upon P&T established criteria and/or Government direction. The P&T Committee will provide PA and/or MN criteria for these agents.

C.8.2. When new clinical information becomes available, the Contractor shall provide for Government consideration, recommendations that are aligned with DoD benefit design rules, pertaining to appropriate use and safety concerns to either update existing or develop new clinical review criteria.

C.8.3. When P&T updates existing PA or MN criteria or establishes new PA or MN criteria, the Contractor shall adopt those changes according to the specified implementation date.

C.8.4. The Contractor shall have a process for validating new clinical review criteria to ensure that claims adjudicate as intended by the Government.

C.8.5. The Contractor shall allow for the portability of PA and MNs approval across all points of service unless otherwise directed by the Government.

C.8.6. Clinical Review Processes

C.8.6.1. The Contractor shall perform the relevant clinical review determination for a PA or MN and transmit the approval or denial to the PDW.

C.8.6.2. The Contractor shall not perform a clinical review if the required PA or MN is active on the system-wide profile, except as described in C.8.10.2.

C.8.6.3. The Contractor shall submit TED records in accordance with the TSM for all approved or denied clinical reviews performed, including those performed electronically per C.11.15.9.

C.8.6.4. The Contractor shall review all supplemental information, as it pertains to the PA and MN criteria, if received with the original PA or MN submission, prior to making a determination for approval or denial of a PA or MN.

C.8.7.4.1. If additional information is received for a denied PA or MN within 30 calendar days of the initial denial, the Contractor's evaluation of the additional information shall be part of the initial review. The Contractor shall not revise the previously submitted TED or generate a new TED for the re-evaluated PA or MN.

C.8.7.5. The Contractor shall allow an MTF pharmacist to report completed clinical reviews and shall document such reviews on the Contractor's system. These PAs shall be immediately available for claims processing.

C.8.7.6. An MTF prescriber, the prescriber's representative, or an MTF pharmacist, may also utilize the Contractor's call center or available tools to submit a clinical review determination, which shall be conducted per the requirements under C.8.10.2. At a minimum, the Contractor must accept the submission of PA or MN via fax, telephone, or ePA.

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C.8.7.7. In cases where the clinical review was conducted by the MTF and placed on the profile by the Contractor (per C.8.7.6 above), the Contractor shall not submit a TED record.

C.8.7.8. The Contractor shall notify the beneficiary and the prescriber of the clinical review decision, except in cases where the review was performed by an MTF under C.8.7.6.

C.8.7.9. For denied clinical review determinations, the Contractor shall notify the beneficiary in writing and advise them of their appeal rights. The initial notification shall contain sufficient information to enable the beneficiary or prescriber to understand the basis for the denial and shall state with specificity what agents are being denied and for what reason (i.e., listing specific PA or MN criteria not met).

C.8.7.10. The Contractor shall perform PA determinations regarding off-label use of agents in accordance with the TPM, Chapter 8, Section 9.1. In addition, the Contractor shall perform coverage determination with respect to rare diseases in accordance with the TPM Chapter 1, Section 3.1.

C.8.7.11. The Contractor shall identify all beneficiaries with an existing PA or MN for specific agents, at the request of the Government.

C.8.7.12. The Contractor shall apply, shorten, extend, or remove a specific PA or MN for any identified patient or group of patients. The Contractor shall execute this process within 30 days of receiving a request from the Government.

C.8.7.13. The Contractor shall process clinical review requests and provide notification to the beneficiary and the prescriber in a manner that meets the minimum processing standards below.

C.8.7.13.1. 98% of all clinical reviews shall be completed and notification sent within three business days of receipt of a properly completed request, measured monthly.

C.8.7.13.2. 100% of all clinical reviews shall be completed and notification sent within 10 business days of receipt of a properly completed request, measured monthly.

C.8.7.14. The Contractor shall provide reporting on clinical review volumes, appeals, and processing times (CDRL Q010).

C.8.7.15. At the Government's request, the Contractor shall provide ad hoc reporting to retrieve PA and MN data extracts by agent or agent class with sufficient detail to determine specific outcomes (approval or denial) for each transaction. The report shall detail decision pathways and specific criteria which lead to approvals and denials. The report shall provide percentage approved or denied via a given pathway over a specified time period. All data extracts must be in CSV or Microsoft Excel® format.

C.8.7.16. The contractor will review every set of PA and MN criteria prior to the start of pharmacy services and at least annually and will facilitate in refining and improving these criteria to include providing recommendations for applicable clinical criteria subject to Government review and approval. Additionally, the Contractor will review and provide a solution to eliminate any redundancies in the clinical criteria to include instances where clinical reviews are combined, such as at the MTFs.

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C.8.7.17. Clinical Reviews for Dispensed as Written prescriptions when generics are available and preferred. The Contractor shall perform PAs using Contractor-developed, Government reviewed criteria to determine when there is a clinical justification to use a brand name agent in lieu of a generic equivalent. The Contractor's criteria and clinical rationale will be made available to the COR, for initial approval and concurrence, not less than 120 days prior to the start of pharmacy services. Once initial criteria are approved, the Contractor may only make changes to the criteria upon the Government's review and concurrence.

C.8.8. Administrative and Automated Overrides. The Contractor shall perform administrative and automated reviews. These reviews do not require the same level of effort as clinical reviews and shall not be considered as such. These reviews include but are not limited to automated overrides for age limit and restrictions based on sex for beneficiaries who meet the criteria, automated profile reviews, as well as quantity limit overrides for vacations, deployments, or agent dosage changes. System-generated overrides shall be distinguished from PAs and MNs resulting from a clinical review on the patient's profile and in the PDW.

C.8.8.1. The Contractor shall not submit a TED for administrative or automated overrides.

C.8.8.2. Administrative Overrides for MTF Claims

C.8.8.2.1. MTF claims processed through the Federal EHR will require PA and/or MN justification when required by P&T. The Contractor shall not allow MTF claims for non-formulary agents from the Federal EHR to adjudicate without documentation of the MN. The Contractor shall return a reject to the MTF when MN is required.

C.8.8.2.2. For non-formulary agents requiring a PA, the Contractor shall address medical necessity through the MN criteria and clinical appropriateness through the PA criteria in a combined presentation per C.8.5.16. Approvals of the combined criteria will be portable to retail and mail order points of service and will allow for a UF cost-share. If a beneficiary has an existing PA approval for a non-formulary agent on their profile, a reject will still be returned to the MTF when a MN approval is not on the profile to initiate a MN review only for approval.

C.8.8.2.3. If a combined MN and PA review is administered by the contractor, a review for MN in addition to a review for PA will also be paid against CLIN X011 or X012 (G.3.5.7.4).

C.8.8.2.4. At the direction of the Government, the Contractor shall load PA and/or MN overrides for existing beneficiaries receiving agents at MTF pharmacies.

C.8.8.2.5. In the case of agents non-compliant with 10 USC 1074g(f) as implemented by 32 CFR 199.21(q), an MN allowing for the dispensing of a non-formulary agent at an MTF shall not allow a patient to qualify for a reduced cost sharing amount.

C.8.8.2.6. In the MTF's support of active-duty service members deployment, the Contractor shall be able to override quantity limits and day supply limits as determined by the Combatant Commands (per C.6.7.6) when requested by the MTFs.

C.8.8.3. Automated Profile Reviews. The Contractor shall provide automated profile reviews (e.g. step therapy, age and sex) for agent classes designated by P&T. The Contractor shall

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perform automated reviews in real-time at all points of service. When a prescription for an agent requiring automated profile review is presented, the automated profile review will look back up to 365 days for qualifying agents in the beneficiary's system-wide profile or for specific profile attributes. The Contractor's look back methodology shall have technical capabilities to be able to address a variety of complex step therapy edits. The Contractor shall not submit TEDs for automated profile reviews.

C.8.9. Prescription Monitoring Initiatives. Under 32 CFR 199.4, TRICARE may not cost share agents to support or maintain a potential abuse situation. In coordination with prescribers and MCSCs, the Contractor shall support prescription and utilization monitoring intended to identify potential abuse situations and restrict access to prevent further abuse. Two aspects of prescription monitoring include the Beneficiary Monitoring Program, described under C.8.12.1, and MTF Restrictions, described under C.8.12.2.

C.8.9.1.1. Appeal Rights and MTF Reconsideration. An appeal of the Contractor's initial determination and any further appeals shall be processed in accordance with the TOM, Chapter 12. The Contractor shall not accept appeals from Active-Duty Service Members (ADSMs). ADSM are not authorized as appealing parties for TRICARE cost-sharing determinations. ADSM should be directed to their local MTF for reconsideration for access to medication issues resulting from a denied clinical review regardless of point of service of the originating prescription.

C.8.9.1.2. All beneficiaries seeking to obtain their prescriptions from a MTF should be notified to return to the MTF for reconsideration upon denial of a clinical review determination.

C.8.9.2. The Contractor shall utilize the beneficiary's preferred method for written communication for a denial notification.

C.8.9.3. Availability of Agents Non-Compliant, 10 USC 1074g(f) as implemented by 32 C.F.R. 199.21(q)(2).

C.8.9.3.1. The Contractor shall limit retail network claims for covered agents non-compliant with 10 USC 1074g(f) as implemented by 32 C.F.R. 199.21(q)(2) and in accordance with TPM, Chapter 8, Section 9.1. The DoD P&T process will determine when covered agents are unavailable through the retail network under this regulation and establish criteria for reauthorization. These items and their respective implementation dates are published on DoD P&T Committee website (Attachment J-3, Line1).

C.8.9.3.2. As the Government restricts access to covered agents non-compliant with 10 USC 1074g(f) as implemented by 32 C.F.R. 199.21(q)(2) the Contractor shall notify beneficiaries with active prescriptions, describing the new restriction and providing information on how to change the current prescription to either an approved agent or move to mail order (See C.9.3.3). The beneficiary may also submit a request for preauthorization, which shall be considered a type of clinical review and processed under the requirements in C.8.1.

C.8.10. Enhanced Coverage Review

C.8.10.1. Consistent with C.11.15.2, the contractor shall provide a solution which enhances

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coverage review by pulling missing or clinically relevant information directly from a prescriber's EHR. The solution will be able to pull data from any EHR that complies with CMS guidelines as defined in CMS-9155-F, as well as any internal sources (e.g. DEERS) that provide data relevant to PA criteria or other types of benefit restrictions as determined by DHA (e.g. quantity limits).

C.8.10.2. The Contractor shall be able to use beneficiary categories and/or Healthcare Delivery Plan (HCDP) codes in the enhanced coverage review capabilities to restrict coverage, bypass clinical review criteria or to present alternative clinical review criteria specific to POS.

C.8.11. Beneficiary Prescription Monitoring Program

C.8.11.1. The Contractor shall manage the Beneficiary Prescription Monitoring Program in accordance with the requirements set forth in TOM, Chapter 28. The Contractor shall coordinate efforts to identify potential candidates with the regional MCSCs and directly with the MTF sites.

C.8.11.2. The Contractor shall generate a quarterly listing of the most likely candidates for restrictions (CDRL Q060).

C.8.11.3. The list will contain the top 300 candidates for each region based on the beneficiary's Primacy Care Manager (PCM) assignment (Prime) or region location (e.g., residential address of Select beneficiaries). All candidates with an MTF PCM will be referred to the respective MTF while patients enrolled with an MCSC (Prime or Select) will be referred to that MCSC to review the candidates for possible restrictions.

C.8.11.4. Upon receiving a determination from an MCSC identifying those beneficiaries to be enrolled in the program, the Contractor shall send them letters in accordance with TOM Chapter 28.

C.8.11.5. Upon receiving the beneficiary's written response designating their prescribing providers, or the MTFs direction on restriction, the Contractor shall lock the beneficiary in accordance with the specified restrictions. Once the beneficiary is locked, the Contractor shall reject all pharmacy prescriptions submitted which violate a beneficiary's restriction(s).

C.8.11.6. The Contractor shall notify the COR and other designated authorities of beneficiaries who have not responded within 14 calendar days to approve the entry of 100% cost share restriction. A list of these beneficiaries shall be provided in CDRL Q060.

C.8.11.7. The Contractor shall post a record of dispensing to the beneficiary's profile when a beneficiary pays full price for restricted agents, since it is considered a non-covered service. The Contractor shall reimburse paper claims submitted by beneficiaries who paid the full cost of a restricted agent, only after authorized by the COR subsequent to a review.

C.8.11.8. The Contractor shall enter the prescriber selections from the beneficiary or restriction guidance from the Government within 24 hours of receiving. Approved overrides to restrictions will be completed as soon as possible, not to exceed four (4) business hours. For one-time exceptions to allow a claim to process, three possible overrides may be offered, depending on the situation and point of service. All overrides shall be reported in CDRL Q061.

- **Override by request** – Immediately upon receiving the request, the Contractor enters an override to allow the prescription to process outside of the restriction. The

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override limit is set per TOM Chapter 28 unless the COR provides additional guidance.

- **Approved Overrides** – When a request is made, the Contractor shall gain approval from the MTF POC or MCSC POC where the restriction originated or the COR as necessary to facilitate the process. The COR will provide approval for MCSC overrides and changes. The Contractor shall enter the override as soon as possible, not to exceed four
- (4) business hours.
- **Pharmacy Overrides** – An override entered by the pharmacy as part of the claim adjudication process.

C.8.11.9. The Contractor shall apply restrictions differently for TMOP, retail, and Federal EHR prescriptions and overrides shall be offered as follows:

C.8.11.9.1. The Contractor shall reject TMOP and retail network pharmacy prescriptions if they are not in compliance with the beneficiary's restriction. An override by request (See C.8.9.3.8 (A) above) may be issued when the beneficiary calls the Contractor. When the number of overrides by request for that period has been reached, the Contractor may approve override (See C.8.9.3.8 (B) above) after approval of the appropriate POC. Federal EHR prescriptions will be rejected if they are not in compliance with the beneficiary's restriction. The pharmacy may enter their own override (See C.8.9.3.8(C) above) or may contact the Contractor to request an override (See C.8.9.3.8 (A) above). Overrides by request shall be honored when requested by the MTF pharmacy.

C.8.11.9.2. The Contractor shall be available during all hours that MTF pharmacies using the Federal EHR are open to assist the MTFs by providing information about the patient restrictions and entering an override by request. The Contractor shall also apply restrictions for beneficiaries at the beneficiary's request (for example, in the case of identity theft). Beneficiaries with restrictions entered at their own request may dis-enroll from the program at any time and remove all restrictions.

C.8.11.10. To aid the Government in monitoring the program, the Contractor shall provide reporting on the number of beneficiaries with restrictions, changes to restrictions over the reporting period, beneficiaries not in compliance with their restrictions, and an MTF compliance summary (CDRL Q061). The Contractor shall provide a monthly updated report to the Government office (each TRO, program office (e.g., dental) or MTF) a list of their assigned beneficiaries restricted in their area (CDRLs M230, M231).

C.8.12. Other MTF Restriction Programs

C.8.12.1. In addition to the Beneficiary Prescription Monitoring Program, individual MTF sites may have specific utilization and prescription monitoring programs, such as Warrior Transition Units, where military personnel may be enrolled for a limited duration. Under these programs, the MTF will make determinations for entry, modification and removal of a beneficiary's restrictions. The MTFs will communicate these determinations to the Contractor to modify restrictions. The Contractor shall enter or update these restrictions as described above.

C.8.12.2. The Contractor shall create and maintain the MTF restriction forms, stored on their website in a location communicated to the MTFs. The Contractor shall provide proposed changes to the Government for review and concurrence. The form will contain, at minimum, the following data elements:

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- New or modified request
- Date of request or modification
- Reinstatements of previous restrictions and effective date
- Restricted beneficiary's information (name, DoD ID, date of birth)
- Type of lock
- Provider(s) name and DEA/NPI
- Pharmacy's name and NPI
- Requestor POC information (phone number, email, signature, and date for prescriber, registered nurse, and/or MTF RPh)
- Restricting MTF Site
- Reason for Request and effective date
- Whether member has been notified of restriction

C.8.12.3. The Contractor shall provide a monthly report containing the beneficiaries' profiles broken out by prescriber (CDRLs M230, M231). This monthly report will also contain at summary level, at a minimum, the following data elements:

- MTF prescribers that write for restricted beneficiaries that are not assigned to them
- MTF prescribers writing a high volume of Schedule II-V prescriptions
- Non-compliant beneficiaries.

C.8.13. Prescription Drug Monitoring Programs

C.8.13.1. The Contractor shall either establish or maintain the MHS as a new "state" for the purposes of sharing data with all other Prescription Drug Monitoring Programs (PDMPs). The Contractor shall be the MHS PDMP administrator. The MHS PDMP will be developed and supported compatibly with PMP Interconnect, the nationwide, interoperable PDMP data sharing hub owned by the National Association of Boards of Pharmacy (NABP). MHS PDMP data shall be available for registered PDMP users for all other states, including prescribers, pharmacists and their respective delegates. The Contractor shall update the PDMP data each day with prescription dispensing activity from the previous day. The data file shall contain all DEA Schedule II-V prescriptions, including compounds and partial fills, which are dispensed from an MTF pharmacy. At the Government's direction, the Contractor shall also include specified non-controlled agents with high potential for abuse. The Contractor shall submit data using the most current version of the American Society for Automation of Pharmacy (ASAP) 4.25A Standard, to include future updates in accordance with industry best practices. The Contractor shall monitor MHS PDMP data on a daily basis and perform validation and reconciliation of the MHS data. The Contractor shall populate the file using each beneficiary's most current available address. As the administrator, the Contractor shall have processes to mitigate data integrity errors and to correct errors as needed to allow record to post to the PDMP. The Contractor shall maintain a log of all these errors and their corrections, which will be made available to the Government for review upon request.

C.8.13.2. The Contractor shall provide MHS registered users with secure web-based access to query the most currently available data from all PDMPs that support sharing with MHS PDMP. The Contractor shall provide support to MHS users for issues related to data content and accuracy of the MHS PDMP database. The Contractor shall register all credentialed MHS prescribers, pharmacists, and their appointed representatives to allow them access to query all

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sharing PDMPs. The Contractor shall facilitate timely registration of all qualified MHS users and provide training and education on appropriate use of the PDMP.

C.8.13.3. The Contractor shall provide monthly reporting on registrations and queries of PDMP data in accordance with CDRL M220.

C.9. Customer Service

C.9.1. Beneficiary Services

C.9.1.1. The Contractor shall offer beneficiary services, including a call center. The Contractor shall operate beneficiary services with personnel predominantly dedicated to this contract and shall respond to beneficiary inquiries 24 hours a day, 365 days a year, in accordance with the contract requirements and performance standards stated below.

C.9.1.2. The Contractor shall provide accurate, complete, and timely responses in a courteous manner to questions from beneficiaries about any aspect of the services provided under this contract through its beneficiary services operation.

C.9.1.2.1. The Contractor shall use best commercial practices and technology that meet the needs of the MHS beneficiary in providing customer support and education resources, including mobile access and social media.

C.9.1.2.2. The Contractor shall monitor and measure social and traditional media presence all public platforms that inform, engage, and influence beneficiary and provider audiences. The Contractor shall track and analyze mentions, reach, and engagement in combination with thematic and sentiment analyses to determine media impact and to inform enhanced, targeted messaging in coordination and collaboration with DCPAD. The Contractor shall produce a quarterly report synthesizing these data-driven findings and trend analyses. Additionally, the Contractor shall monitor beneficiary survey results to identify and flag any emergent beneficiary issues that could impact and drive social media content developed by the DCPAD team to drive responsive and agile beneficiary communication.

C.9.1.2.3. The Contractor will implement and support Telepharmacy using Contractor Offered Services, Methodologies, and Techniques described at C.16.6.

C.9.1.2.4. The Contractor shall provide beneficiary services to beneficiaries with special needs. The Contractor shall provide beneficiary services to non-English speaking beneficiaries.

C.9.1.2.5. The Contractor shall provide beneficiary services for the hearing and visually impaired TRICARE beneficiaries.

C.9.1.3. The Contractor shall offer toll-free number(s) and number(s) for international callers in support of all the services provided under this contract, based on the requirements in TOM Chapter 23, Section 4 and Chapter 11, Section 6.

C.9.1.4. The Contractor shall maintain a list of all telephone and fax numbers used in the support of this contract and shall provide updates to the Government when numbers are added or changed and upon request.

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C.9.1.5. The Contractor shall provide beneficiaries with access to their own claims history for no fewer than 18 months after their TRICARE eligibility has ended.

C.9.1.6. When the Contractor cannot resolve a specific beneficiary issue related to care not covered under this contract, the Contractor shall facilitate either a warm transfer to internal organizations when appropriate and approved by the Government or provide the beneficiary contact with the appropriate organization to seek additional guidance, in accordance with any Memorandum of Understanding (MOUs). This requirement includes, but is not limited to, the following situations:

- If the beneficiary's issue concerns DEERS or DEERS eligibility, the Contractor shall refer the caller to the DMDC Beneficiary Telephone Center and provide phone number and hours of service. These numbers are for beneficiary use only and cannot be used by the Contractor or other service provider.
- If the beneficiary's issue concerns coverage administered by another DHA Contractor (e.g., the MCSCs for their region), the Contractor shall refer the beneficiary to the appropriate TRICARE Contractor. The Contractor shall continue to work the issue until resolved or otherwise dispositioned.

C.9.1.7. The Contractor's automated response system shall offer an option for beneficiaries to check the status of their mail order agent prescription orders. The initial menu shall also offer beneficiaries the option of being immediately transferred to a Customer Service Representative.

C.9.1.8. At the beneficiary's request, the Contractor shall perform real-time coverage checks to verify whether a prescription will process under the benefit and confirm the copay that will be assessed. The Contractor's system shall perform a comprehensive mock adjudication utilizing the complete beneficiary profile and claims history when performing the coverage check. The outcome of the coverage check shall accurately reflect how a claim would adjudicate at that time, including:

- Availability and applicable copays at different points of service
- Existing clinical reviews on file and applicable lookback and bypass logic
- Claims history at all points of service
- Benefit rules and formulary limitations

C.9.1.9. The Contractor shall monitor issues driving call center volume and provide a monthly report of the top call center issues to the Government (CDRL M020). Additionally, significant issues that drive high call volumes or other significant sources of beneficiary dissatisfaction shall be reported to the COR or other designated authorities. Interim updates on specific issues shall be provided to the Government upon request.

C.9.1.10. The Contractor shall monitor beneficiary calls and industry trends to identify emerging issues impacting TRICARE beneficiaries on an ongoing basis. These issues shall be communicated to the COR or other designated authorities on a timely basis. The Contractor shall work in collaboration with the Government to address these issues as appropriate.

C.9.1.11. The Contractor will utilize the Customer Service Support Tool described as a part of Contractor Offered Services, Methodologies and Techniques at C.16.7 in supporting this task.

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C.9.1.12. To streamline call monitoring and audits, the contractor shall provide the Government with dedicated remote and on-site capabilities for each help desk that enable re-review and analysis, such as recordings and transcripts as described in TOM, Chapter 11, Section 6.

C.9.1.13. The contractor shall ensure that Government auditors have sufficient information to confirm that validation occurred consistent with HIPAA requirements (see C.12) and to conduct any necessary follow-up for specific beneficiaries or claims that were identified during monitoring.

C.9.1.14. The Contractor shall obtain the necessary access to accept referred customer service cases via the Government's secure, web-based Assistance Reporting Tool (ART) which promotes customer service by facilitating beneficiary case resolution with less risk of compromising Protected Health Information (PHI).

C.9.1.15. All written responses to beneficiaries shall meet the standards established in the Plain Writing Act of 2010 (See 5 USC 301) as implemented in DoDI 5025.13, communicating to beneficiaries in a manner that is "clear, concise, well-organized and follows other best practices appropriate to the subject or field and intended audience." The Contractor shall use alternative government identification numbers, such as the DoD Benefits Number (DBN) in place of the SSN on outgoing correspondence from the Contractor to the beneficiary.

C.9.1.16. The Contractor shall monitor priority correspondence (See TOM Chapter 23, Section 4) addressing any beneficiary issue under this contract received from any source and provide reports of priority correspondence updated as correspondence is entered or closed. The Contractor shall forward priority correspondence to the Government in accordance with TOM Chapter 11, Section 5. In lieu of the reporting requirements in TOM, Chapter 11, Section 5, the Contractor shall follow the reporting requirements in CDRL M030.

C.9.1.17. Consistent with TOM Chapter 23, Section 4, the Contractor shall provide an explanation of benefits (EOB) to beneficiaries who obtain pharmacy services. The Contractor shall provide EOBs primarily by electronic means but will also offer delivery by mail at the beneficiary's request or if a valid email address is not available.

C.9.1.18. The Contractor shall generate Electronic EOBs monthly but mailed EOBs will only be provided on a quarterly basis. Quarters are defined by the calendar year. EOBs will be generated no later than 11 days after the end of the period being reported (both monthly and quarterly). Electronic EOBs shall contain information on prescriptions dispensed at all points of service, excluding only those referenced under C.9.1.5.4.

C.9.1.19. Mailed EOBs shall be sent only to those beneficiaries receiving prescriptions through retail or mail order points of service. If the beneficiary also receives prescriptions from additional points of service during that time-period covered by the EOB, those shall be listed as well, except for those referenced under C.9.1.5.4.

C.9.1.20. The Government may exclude other POS or specific programs from EOBs.

C.9.1.21. For MTF and MOP claims appearing on an EOB, the Contractor shall apply pricing from the medical pricing file based on the lowest cost entry within the GCN.

C.9.1.22. The Contractor shall track the volume of EOBs sent through both channels, including the

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number of returned electronic notifications, the number of beneficiaries that accessed their electronic EOB and the number of mailed copies that were returned (CDRL Q080).

C.9.1.23. As measured monthly, the minimum performance shall be as follows:

Beneficiary Help Desk	
Service Category	Performance Standard
Average Speed of Answer (measured from initial connection to answer by a service representative - includes any introductory message or automated menus)	60 seconds or less
Telephone Call Blockage rate	5% or less
Abandoned Call rate at any point	5% or less
Telephone Calls Resolved at any point	95% during initial call, 100% within 2 days
Priority Correspondence - Complete and issue resolved (to the Government's satisfaction, includes Electronic)	85% during 10 days, 100% within 30 days
Routine Correspondence (Includes Electronic)	85% within 15 days, 100% within 45 days

The Contractor shall provide reporting on all metrics (CDRL Q020).

C.9.2. Pharmacy Help Desk Service

C.9.2.1. Starting no later than 40 days prior to the start of pharmacy services, the Contractor shall provide a dedicated toll-free number for a pharmacy help desk that helps retail network pharmacies provide courteous, prompt, and efficient retail pharmacy services to TRICARE beneficiaries in accordance with TRICARE Pharmacy Benefits Program requirements.

C.9.2.2. The Contractor shall also accommodate calls from MTF pharmacies to support processing of claims, dispensing of agents, or other related issues. The Contractor shall provide a dedicated toll-free number in support of MTF pharmacies and shall have staff specifically trained to support the MTFs. The Contractor shall respond to inquiries from retail network pharmacies and MTF pharmacies 24 hours a day, 365 days a year, in accordance with the performance standards stated below.

Pharmacy Help Desk	
Service Category	Performance Standard
Average Speed of Answer (measured from initial connection to answer by a service representative)	60 seconds or less
Telephone Call Blockage rate	5% or less
Abandoned Call rate at any point	5% or less

C.9.2.3. The Contractor shall provide call monitoring capabilities in accordance with C.9.1.1.12 0 for the MTF Pharmacy Help Desk.

C.9.2.4. The Contractor shall report MTF calls separately from those from retail pharmacies.

C.9.2.5. The Contractor shall provide reporting on all metrics (CDRL Q030).

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C.9.2.6. The Contractor shall provide a pharmacy-specific website to communicate important and timely information pertinent to retail network and MTF pharmacies. Information shall include but is not limited to: State of Emergency processes, formulary guides, appeal guidelines, and how to troubleshoot common adjudication rejects.

C.9.3. Beneficiary Notifications

C.9.3.1. Unless otherwise specified, the Contractor shall send all written notifications to beneficiaries required under this contract either electronically or by first class mail. Notifications not required to be in writing may utilize the communication methods listed above and additionally, phone and text message. As technology evolves, the Contractor may propose alternative communication channels to the Government for consideration.

C.9.3.2. Beneficiaries shall have a means to indicate their preferred communication method to the Contractor. The Contractor shall follow the beneficiary's indicated preference unless it is determined to be invalid (e.g., undeliverable mail/email) or otherwise superseded by the contract.

C.9.3.3. The Contractor shall use beneficiary preferred effective methods (e.g. email, smart phone applications) to send notices to newly-eligible beneficiaries, as identified by the Government on a quarterly basis. The notifications shall contain, at a minimum:

- A brief description of the TRICARE Pharmacy Benefits Program.
- The Contractor's contact information, including mailing address, beneficiary service telephone numbers, toll-free numbers for overseas beneficiaries, and the Contractor's email addresses.
- The Contractor's TRICARE Pharmacy Benefits Program website address.
- Contractor supplied mail order registration form
- Information on how the beneficiary can download the TRICARE Pharmacy Benefits Program Handbook from the Contractor's website.

C.9.3.4. The Contractor shall notify beneficiaries who, within the past year, have received prescriptions for agents that are newly-designated as (A) non- formulary; (B) for restricted access at retail due to non-compliance with Federal Ceiling Price (FCP);(C) requiring prior authorization or step therapy; (D) not eligible for automatic refill program; and (E) non-covered. Formulary and FCP compliance decisions are available on the DoD P&T Committee website on a quarterly basis (Attachment J-3, Line 1). The DoD P&T Committee may also identify other situations requiring notification to beneficiaries.

C.9.3.5. In each of the beneficiary communications listed in C.9.3.3, the Contractor shall identify beneficiaries selected to receive the notification. The Contractor will work with the Government to understand the parameters (e.g. drug and beneficiary identifiers) and frequency of the communications (e.g. pre and/or post implementation) which may differ given the scenarios listed in C.9.3.3. The data elements to be queried for each mailing will be provided to the Government for review and concurrence prior to the actual data pull.

C.9.3.6. The Contractor shall ensure that notices clearly explain the benefit design change(s) and identify formulary alternatives and copays, as well as any additional information required to ensure continuity of care. These notices shall be approved prior to mailing by the COR or KO.

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C.9.3.7. At the direction of the CO, the Contractor shall provide notifications to beneficiaries identified by the Government regarding changes to the prescription agent benefit or other prescription agent information. The Contractor shall ensure that these notices are mailed to beneficiaries within five calendar days of receiving direction from the CO. The notice shall be approved by the COR or other designated authority prior to being mailed.

C.9.3.8. The Contractor shall monitor clinical issues agent recalls and market withdrawal notices and notify beneficiaries who have filled impacted agents at retail, mail order or MTF pharmacies, notifying them of these issues. The Contractor will provide a template of the communication to the Government for approval.

C.9.3.9. Prior to sending out any mailing under this contract, the Contractor shall utilize commercially available mailing preparation software to scrub beneficiary mailing addresses.

C.9.3.10. The Contractor shall monitor returned mail and shall not continue to send mail to beneficiaries with known bad addresses.

C.9.3.11. When a valid email address is available and the beneficiary has indicated a preference for electronic communications, the Contractor may issue any notification by email, except where noted otherwise. Collection, maintenance, and use of email addresses shall be in accordance with the Contractor's plan described in C.9.4.1.1.6.1.

C.9.3.12. The Contractor shall monitor undeliverable email and will not continue to send messages to known invalid email addresses.

C.9.3.13. If the Contractor is notified that emails are being received by someone other than the intended recipient, the Contractor shall discontinue use of the email address until it has been verified by the beneficiary and corrected.

C.9.3.14. All communications with beneficiaries are subject to review by the Government upon request. The Contractor shall electronically provide the Government with copies of all mailings to be distributed to beneficiaries.

C.9.4. Education and Outreach

C.9.4.1. Beneficiary Education

C.9.4.2. The Contractor shall submit an annual comprehensive beneficiary and healthcare provider education plan to the Government, outlining how it intends to conduct targeted TRICARE Pharmacy Benefits Program education to beneficiaries, healthcare providers, and other stakeholders identified by the Government, as described in TOM, Chapter 11, Section 1 (CDRL A100). Issues may be identified by the Government or the Contractor. Additionally, the plan shall meet the following minimum requirements:

C.9.4.2.1.1 Establishes goals for educational plan and metrics to evaluate performance relative to these goals.

C.9.4.2.1.2 Provides monthly updates, social media messages, smart phone application updates, news articles, graphics, infographics, videos or other items of interest, of current best practice or using the

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latest platforms and channels of communication to the DCPAD as determined in the Memorandum of Understanding (MOU).

C.9.4.2.1.3 Includes a plan on how educational materials, letters, and other educational outreach to beneficiaries and healthcare providers will be delivered, such as by use of email, text, video, smart phone application, web-based programs, U.S. Mail, or any latest platforms and channels of communication.

C.9.4.2.1.4 Includes a plan for how the Contractor will acquire email addresses and maintain them, recognizing that any Contractor collection of email addresses must have appropriate disclaimers to advise the beneficiary of how this PII will be protected.

C.9.4.2.1.5 Includes a plan describing its sustained communication effort to educate the beneficiary population about the benefits of receiving electronic EOBs and to influence greater adoption of their use.

C.9.4.2.1.6 Include a plan which describes how the Contractor will:

- Minimize beneficiary confusion with respect to website and electronic applications for accessing agents profiles at designated mail order pharmacies particularly if the website/applications differ between these pharmacies.
- Route or connect the beneficiary to the appropriate pharmacy which can dispense or provide information on their agent(s) if beneficiary outreaches to the Contractor telephonically or through electronic means.
- Inform beneficiaries and providers on which network pharmacies can dispense their prescriptions to include limited distributions agents, agents enrolled in Expanded Use of MTF and TMOP or restricted to a POS by policy.
- Inform beneficiaries and providers on the benefits of utilizing the different points of service.
- Minimize beneficiary confusion by standardizing prescription labels for agents dispensed from different TMOP facilities.

C.9.4.2.2. All Contractor educational content shall use Plain Language in accordance with DoDI 5025.13.

C.9.4.2.3. The Contractor will provide input on content developed by DCPAD within five business days. Reviews will include printed products, web and social media content, responses to queries for media requests, customer service scripts/training manuals/curriculum (e.g., TRICARE Fundamentals Course), frequently asked questions, senior leader talking points, briefings, and other products identified by the Government.

C.9.4.2.4. The Contractor will work closely with DCPAD Communications to ensure all TRICARE beneficiaries and other stakeholders identified by the Government, receive unified, timely, accurate, consistent, and effective products and tools that improve their access, understanding, and use of the TRICARE Pharmacy Benefits Program. All educational materials and communications content, hereafter referred to as content, will be coordinated with DCPAD. DCPAD will review and approve content developed by the Contractor.

C.9.4.2.5. The Contractor shall provide educational content on issues relating to the TRICARE

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Pharmacy Benefits Program for use by the Contractor and the Government, in accordance with CDRL Q160.

C.9.4.2.6. The Contractor shall maintain and update a content calendar in collaboration with DCPAD and POD SMEs that meets the needs of the Government and ensures appropriate lead time for review, finalization, and dissemination

C.9.4.2.7. The Contractor's timeline for submission shall include sufficient time, as determined by the Government, review for audience appropriateness, accuracy, timeliness, use of plain language, 508 compliance, and any other Government criteria identified by the Government. DCPAD and the Contractor will work closely to address completion of items with a short suspense.

C.9.4.2.8. All content provided by the Contractor shall contain accurate, original, and publication-quality content that requires minimal editing by the Government, use of plain language, 508 compliance, and any other Government criteria identified by the Government.

C.9.4.2.9. The Contractor shall submit all content to the COR, DCPAD or other designated authority for review and final approval prior to distribution (see beneficiary communication templates/SOP at C.14.13) to internal government partners or MCSCs as well as external partners and the public. All publications shall be in accordance with relevant DHA and MHS Style and Branding Guides, including those for TRICARE and the Federal EHR.

C.9.4.2.10. The Contractor shall attend the annual DCPAD Training Conference which runs approximately three days, and participate in monthly DCPAD Partnership Meetings, both are held in the National Capitol Region (NCR). The Contractor shall provide representation that can address all issues involving beneficiary and network provider education and communication to include print, web, social media, customer service, and media relations.

C.9.4.2.11. The Contractor shall participate in ad hoc meetings to address communication and education activities in support of the TRICARE Pharmacy Benefits Program, to include print and web content. Meetings shall be attended via teleconference or video telecommunications as directed by the Government. The purpose of these meetings are to develop, review, or revise core educational materials, discuss observations and possible changes in education and communication activities. The meeting participants are tasked with reviewing current policies and procedures to determine where proven best practices from government and private sector operations can be implemented in the administration of TRICARE Pharmacy Benefits Program to continue TRICARE's leading role as a world class health care delivery system.

C.9.4.2.12. The Contractor shall attend the annual Joint Federal Pharmacy Seminar and MHS Conference to participate in joint educational efforts.

C.9.4.2.13. The Contractor shall alert the COR and DCPAD about news that will have impact on the beneficiary population and is likely to increase customer service or media contacts. The Contractor shall conduct a media-relations program in accordance with C.9.1.1.2.1, Department of Defense guidelines (DoD Directive 5122.05 and DoD Instruction 5410.20) and guidance provided by DCPAD. The Contractor shall speak only on issues for which it has direct responsibility in the administration of the TRICARE Pharmacy Benefits Program. All questions beyond the Contractor's scope shall be referred to DCPAD.

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C.9.4.2.14. The Contractor shall ensure all published materials communicate consistent with TRICARE program messages that employ TRICARE's mission and vision, as detailed on the TRICARE website. After approval from the Government, the Contractor shall perform the following unless otherwise directed:

- Respond to media questions about its role as a contractor, responsibilities, and actions on behalf of the Government in support of the TRICARE Pharmacy Benefits Program.
- Follow-up media contacts by sending copies of information provided to the media and a summary of any discussions to DCPAD when immediate action was required.

C.9.4.2.15. Use of printed materials will be limited to essential products, and the Contractor shall assist the Government in identifying the most cost-effective and efficient delivery of beneficiary educational materials. The Contractor is responsible for all storage, handling, and distribution of printed materials that are produced and shipped to the Contractor.

C.9.4.2.16. The Contractor shall distribute printed materials to individuals, MTFs, Beneficiary Counseling and Assistance Coordinators (BCACS) or other entities, as requested. The Contractor shall accept and fulfill orders for printed materials from designated POCs submitted via the Contractor's link on a government website. The Contractor may request additional printed materials from the Government on a quarterly basis.

C.9.4.2.17. The Contractor shall accept requests from beneficiaries to opt-out of receiving educational materials by mail, email, or text. The opt-out will not apply to notifications pertaining to safety and recall issues, benefit design changes (e.g., formulary changes) or the processing of specific claims or clinical reviews. In the event the beneficiary opts out of text messages, the Contractor shall utilize another channel (e.g., email, U.S. mail) to provide the required information to the beneficiary.

C.9.4.2.18. Agent Disposal. There are several safety concerns associated with patients having unused and expired agents in the home. The Contractor shall promote beneficiary safety by providing educational support to beneficiaries on the proper handling and disposal of unused or expired agents.

C.9.4.2.19. Prescriber Outreach. The Contractor shall engage in outreach to medical providers via the TRICARE MCSC contracts. Efforts shall include education on the TRICARE formulary and benefit design, PA submission, real-time prescription benefit check, and other relevant topics. The Contractor shall make educational materials available to prescribers as part of this outreach.

C.9.5. Website

C.9.5.1. All published web content shall be 508 compliant, per 29 USC 794d and overall public guidance (see Attachment J-3, Line 9). The contractor shall submit all content to the COR, DCPAD or other designated authority for review and final approval prior to posting (see beneficiary communication template/SOP referenced at C.14.13).

C.9.5.2. The Contractor shall provide a Health Insurance Portability and Accountability Act (HIPAA) compliant website in support of the services provided under this contract.

C.9.5.3. The Contractor's website shall meet the applicable accessibility standards at 36 CFR 1194.

C.9.5.4. The contractor shall support authentication using MyAuthDS Logon as described in TSM,

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Chapter 1, Section 1.2. In addition to meeting the minimum requirements established within this contract, the Contractor shall ensure that its website and any mobile tools are consistent with commercial best practices and offer features, information, and functionality no less than those available to the Contractor's commercial clients.

C.9.5.5. The Contractor shall maintain a development/test environment to facilitate Government review and approval of mock content, such as website changes and beneficiary correspondence.

C.9.5.6. At minimum, the Contractor's website shall offer the following information and functions:

- Provide a description of the TRICARE Pharmacy Benefits Program;
- Provide Contractor contact information, including phone and fax numbers, mailing, and email address(es);
- Provide an email link to allow beneficiaries or other interested parties to contact Contractor by email with inquiries or comments;
- Allow beneficiaries to register online to use TMOP and provide downloadable forms for TMOP registration and prescription ordering;
- Allow TRICARE beneficiaries to manage their TMOP account(s) to include order refills, track their prescription status, postpone prescriptions, view, release for shipping or cancel existing postponed prescriptions, and update shipping address;
- Show the current status of all prescriptions or claims submitted;
- Allow TRICARE beneficiaries to check the status of member submitted (DMR) claims filed for services provided through a retail pharmacy;
- Provide the ability to locate TRICARE retail network pharmacies, at minimum by zip code;
- Provide PA and MN forms and criteria and make them available for downloads;
- Allow TRICARE beneficiaries to download and print an EOB detailing the beneficiary's retail, TMOP, and MTF prescription activity in accordance with the TOM, Chapters 8 and 23, providing prescription activity for the preceding 27 months at a minimum;
- Provide a link to the TRICARE website to allow beneficiaries to download and print the DD2642 claim form;
- Provide links to online agent and health information;
- Provide links to the TRICARE pharmacy website and Regional MCSCs' websites;
- Provide a real-time web-based formulary search tool as described in C.9.4.9. and
- Contractor will provide care giver access for beneficiaries requiring assistance with managing prescription needs. Caregiver access will be streamlined in the registration process.

C.9.5.7. The Contractor will maintain a responsive website and all portions of the Contractor's website will present information that is machine readable, in mobile-optimized format, and include Web APIs where applicable and in accordance with the May 23, 2012 Federal CIO Memorandum.

C.9.5.7.1. The Contractor shall not duplicate benefit information on the Contractor's website that already exists on TRICARE.mil and will embed links throughout its site to take beneficiaries back to the TRICARE.mil website for this content. The Contractor shall work closely with DCPAD to identify appropriate linkages and content for use on their site. The Contractor shall provide static links to web content referenced on DHA websites.

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C.9.5.8. The Contractor shall make any information or resources not covered by Privacy or HIPAA regulations accessible without requiring an account registration or login.

C.9.5.9. Formulary Search Tool. The Contractor shall provide a real-time web-based Formulary Search Tool (FST) available for public access that accurately reflects the most current benefit design.

C.9.5.9.1. The Contractor shall document changes made to the FST and the expected outcomes. Documentation will include formulary status, PA or MNs, notes, and system changes, etc. Information will be provided to the Government on a regular basis. (CDRL M200)

C.9.5.9.2. The Contractor shall ensure that all FST content (e.g., formulary status, BCF/ECF, PA's, MN's, age, and restrictions based on sex, etc.) accurately reflects the implemented TRICARE benefit design in real time. Accuracy is based on the intent of the DoD P&T Committee.

C.9.5.9.3. The Contractor shall correct 99% of FST content errors within two business days of the inaccuracy being identified.

C.9.5.9.4. The Contractor shall correct 100% of FST content errors within 10 business days of the inaccuracy being identified.

C.9.5.9.5. The Contractor's FST shall include but is not limited to the following items:

- Identify agent (generic or brand) name, strength and formulation;
- Provide general agent information;
- Allow searches by generic name, brand name (trade name) and chemical name;
- Utilize a commercial standard predictive search algorithm which accounts for common misspellings;
- Allow results filtered by strength, form, route and agent type (brand or generic);
- Show status based on Uniform Formulary rules and coverage rules (non-covered) and availability at each POS (i.e., agents restricted by POS);
- MTF Basic Core Formulary (BCF) and Extended Core Formulary (ECF);
- Show associated cost-sharing amounts based on coverage statute and POS;
- Show any restrictions, including but not limited to mandatory generic requirement, age restrictions, restrictions based on sex, quantity level limits, PA, and MN or step-therapy; Note agents that are only available through specific points of service or mandated policies such as agents enrolled in the Expanded Use of MTF and TMOP program, specialty agents (C.4.1), Home Infusion Therapy program (HIT);
- Provide access to any forms associated with the above restrictions;
- Have the ability to show special messaging as provided by the Government, at least 300 characters in length;
- List formulary alternatives based on Uniform Formulary (UF) classes;
- Provide status if FST is under maintenance or users are experiencing difficulties, including the type of issues occurring and expected resolution date;
- Be accessible to the public without requiring registration or login, using a static link.

C.9.5.9.6. The Contractor shall provide an email address on the FST page to allow users to

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provide feedback and ask questions. Feedback shall be made available to the Government upon request.

C.9.5.9.7. The formulary search tool shall be designed to be easily used and understood by the beneficiary, requiring as few steps as possible to navigate to the desired information

C.9.5.9.8. The Contractor shall update the tool to reflect benefit design changes immediately on their effective date.

C.9.5.9.9. The formulary search tool shall have the ability to host formulary lists such as but not limited to specialty drug list, PA/MN list, non-formulary, drug with quantity limits, expanded mail order and MTF drug list, and vaccines. The Contractor shall maintain and publish these lists as updates are made to the benefit design. These lists should be accessible to end-users who wish to download these lists from the formulary search tool in a format mutually agreed upon by the Government.

C.9.6. MTF Education and Support

C.9.6.1. MTF Education

C.9.6.1.1. The Contractor shall provide regular education sessions to MTF prescribers and pharmacies, including similar outreach to that described in C.9.4.3.

C.9.6.1.2. The Contractor shall provide a reject guide to MTF pharmacy staff that provides detailed information on rejects returned to the Federal EHR, including the reason for the reject and any available steps to override the reject or correct the claim. This guide shall be made available for download on the website described in C.9.5.1.3 and updated as required to address frequent questions or changes to rejects or reject handling.

C.9.6.1.3. The Contractor shall provide a website specifically for MTF providers to communicate with the Contractor and to reduce reliance on phone calls and/or faxes. The website shall support the following:

- Authentication via Common Access Card (CAC) or My Auth
- Submission of completed PA forms (for use if ePA is not available or can't be used)
- Facilitate communication with providers - such as to clarify MOP prescriptions, including specialty
- Make various forms and training materials available for download (eg, PMP, DPP)
- Hosting of MTF PMP reports for download (see C.8.9.3.12)
- Receipt of prescriptions, both initial and renewals
- Substitute for faxes, which should not be used to communicate with MTF prescribers
- Access for delegates of prescribers to perform functions
- DPP, described in C.6.7.
- 24/7 technical support

C.10. Claims Reviews and Audits

C.10.1. Claims Research and Corrections

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C.10.1.1. The Contractor shall research any claim, regardless of date of service, at the request of the Government.

C.10.1.2. The Government reserves the right to direct audits of retail or mail pharmacies. In addition to any Contractor initiated on-site audits for which TRICARE is the primary focus of the audit, the Contractor shall perform up to five Government-directed on-site audits per option period.

C.10.1.3. The Contractor shall be able to generate corrected retail transactions when the pharmacy is unable to reverse and/or edit the claims themselves.

C.10.1.3.1. These claims shall be distinguishable from pharmacy self-corrections and are not billable for additional administrative fees. The Contractor shall not identify or submit these corrections as paper claims.

C.10.1.4. Claim research and disputes shall result in corrected claims and the government made financially whole. All claims shall be resolved according to benefit design and formulary restrictions in effect on the date of service. The Contractor shall perform all necessary research and will resolve all discrepancies for each claim identified within 60 days from the date of identification.

C.10.1.5. The Government may withhold the agent cost, dispensing fees, and administrative fees from future administrative fee payments for any research not completed and resolved within 60 calendar days. Withholds will apply to the specific claims that caused the audit to be initiated.

C.10.1.6. Any discrepancies identified by the Government in the monitoring of this contract shall be subject to Contractor desktop audits and, if necessary, on-site audits at the direction of the Government.

C.10.1.7. The Contractor shall perform offsets or recoupments of any identified discrepancies for claims during the performance of services of this contract and predecessor contracts in accordance with TOM, Chapter 10.

C.10.1.8. All requirements listed under C.10.1 shall apply to the specific research areas outlined below.

C.10.1.9. The Contractor shall perform research to facilitate the Government's response to issues by and on behalf of beneficiaries.

C.10.1.9.1. The Contractor shall provide an initial response to requests to research specific beneficiary issues within four (4) business hours or no later than four (4) business hours into the next business day.

C.10.1.9.2. If this initial answer does not contain a complete response, the Contractor shall offer an estimated timeframe for how long it will take to fully research the issue. If further action is required to resolve the issue, the Contractor shall provide an estimated timeframe for resolution. The Contractor shall track research requests and beneficiary issues, including the date requested and resolved.

C.10.1.10. The Contractor shall resolve manufacturer disputes of the TRICARE utilization data, including but not limited to 340B and Medicaid Subrogation claims (where this information is available / data fields are populated) within the past ten years. This includes initial submission of

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claims, and any adjustments, corrections, or cancellations necessary for claims processed.

C.10.1.10.1. The Contractor shall participate in quarterly meetings to review the status of manufacturer disputes

C.10.1.10.2. The Contractor shall submit a report on the status of manufacturer disputes (CDRL M240).

C.10.1.11. Claims research shall be performed to provide additional information on potential issues in the claims adjudication process.

C.10.1.12. Program Integrity Daily Claims Review

The Contractor shall perform an automated review of 100% of all claims daily. Data analysis includes, but is not limited to:

- Establish baseline data to enable TRICARE to recognize unusual trends, changes in agent utilization over time, physician referral or prescription patterns, and plan formulary composition over time;
- Analyze claims data to identify potential errors, inaccurate TROOP accounting, and pharmacy billing practices and services that pose the greatest risk for potential fraud, waste and abuse to the TRICARE program;
- Identify items or services that are being over utilized;
- Identify problem areas within the plan such as enrollment, finance, or data submission;
- Identify problem areas at the pharmacy and prescriber level;
- Compare claims information against other data (e.g., prescriber, agent provided, diagnoses, or beneficiaries) to identify potential errors and/or potential fraud and abuse; and
- Use findings to determine where there is a need for a change in policy.

C.10.1.13. The Contractor shall add and refine parameters on an ongoing basis to adapt to industry trends and areas of concern, applying knowledge and lessons learned from TRICARE as well as other government and commercial payers.

C.10.1.14. The Contractor will submit a monthly report showing audit findings, status of all claims in research, outcomes of completed research, and status of offsets or recoupments (CDRL M090). This report shall include all retail and mail order claims.

C.10.1.15. The Contractor shall develop and maintain a Fraud and Abuse Monitoring and Auditing Work plan that meets the requirements established in TOM, Chapter 13. The plan will also include, but is not limited to, the audits described below.

C.10.1.15.1. The Contractor shall take the following action when inappropriate billing practices at the pharmacy level occur when pharmacies engage in activities including but not limited to the following:

- Incorrectly billing for secondary payers to receive increased reimbursement
- Billing for NDCs that were not dispensed
- Billing for incorrect quantity or days supply
- Billing for non-existent prescriptions

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- Billing multiple payers for the same prescriptions, except as required for coordination of benefit transactions
- Billing for brand when generics are dispensed
- Billing for prescriptions that are never picked up (i.e., not reversing claims that are processed when prescriptions are filled but never picked up)
- Inappropriate use of DAW codes
- Prescription splitting to receive additional dispensing fees
- Agent diversion
- Controlled substance prescriber utilization — top prescribers state/region/national Beneficiary Submitted Claims (high aggregate dollars, network pharmacy)
- Morphine Milligram Equivalents (MME) > 90 per day
- Inappropriate compounding practices

Notwithstanding TOM, Chapter 13, Section 1, Paragraph 1.4.1, as a result of its fraud and monitoring efforts, the Contractor shall refer to DHA Program Integrity a minimum of six cases per option period. Each case will involve a loss of \$100,000 or greater per case to the Government without patient harm, or any case involving patient harm. The Contractor shall provide a Fraud and Abuse Summary Report on the activities outlined in this Section and TOM, Chapter 13 (CDRL Q100).

C.11. System Interfaces

C.11.1. The Contractor shall maintain a number of system interfaces to support the requirements under this contract. All interfaces shall include:

C.11.1.1. Applying and maintaining appropriate functional upgrades and security measures;

C.11.1.2. Providing and maintaining required test environments;

C.11.1.3. Monitoring connections and performing required troubleshooting; and

C.11.1.4. Executing and maintaining full system documentation with interface partners.

C.11.1.5. The Contractor shall support any agreements, documentation, and operational processes required to establish working relationships with interface partners, including but not limited to MOU, Data Sharing Agreements, and issue reporting mechanisms.

C.11.2. The Contractor shall support the use of the DoD authentication service, as described in TSM, Chapter 1, Section 1.2, and CAC logins whenever possible. This includes beneficiary, prescriber, and pharmacy facing systems.

C.11.3. Interface Control Documents

C.11.3.1. The Contractor shall maintain Interface Control Documents (ICDs) for all system interfaces, except in cases when such interface specifications are maintained by the Government (eg, DMDC Interfaces). ICDs shall be provided in a non-proprietary format and shall utilize system names in lieu of specific companies that may currently hold those contracts. They shall be maintained through the mechanism in C.14.13.

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C.11.4. B2B Connection and VPN Devices

C.11.4.1. The Contractor shall complete all tasks related to the documentation and implementation of Business-to-Business Gateway (B2B) telecommunications links. Tasks include but are not limited to:

C.11.4.1.1. Coordination among the Pharmacy Operations Division (POD), DHA Chief Information Officer, MTFs, and other contractors as required. Completion and approval of all B2B required documentation, including CDRL160 (B2B Gateway Questionnaire).

C.11.4.1.2. Procurement and configuration of the Virtual Private Network (VPN) and related equipment.

C.11.4.1.3. Initiation and completion of all required testing and implementation activities.

C.11.4.1.4. Support for all required follow-on activities, including updating documentation, performing IP address and port changes, performing security updates and executing related configuration changes.

C.11.5. DMDC Interfaces

C.11.5.1. The Contractor shall interface with DMDC interfaces and applications, including DEERS, to verify eligibility, update the CC&D Database (CCDD) file, obtain demographic data for the PDW, and update beneficiary contact information. DMDC interfaces are utilized during the claims adjudication process, post adjudication processes, and via web-based applications. The Contractor shall use the applications described in the TOM and TSM. Documentation for DMDC interfaces is maintained by the Government.

C.11.5.2. The Contractor shall participate in regular integration testing meetings, as directed by the Government, throughout the duration of the contract. These meetings may occur as often as daily during contract transition but will likely move to two occurrences per week thereafter.

C.11.5.3. The Contractor shall support user acceptance testing and regression testing to support updates and maintain accounts with specified systems for entering and logging issues and defects.

C.11.6. Connections to Other Federal Pharmacies

C.11.6.1. The Contractor shall connect to DVA, PHS, and IHS pharmacies identified by the Government and support online adjudication for claims received from these pharmacies, in accordance with the individual agency pharmacy claims adjudication rules. The Contractor shall connect to these pharmacies in the same manner as retail network pharmacies (Note: U.S. Coast Guard sites listed in Attachment J-6 are considered to be MTFs and are included under Federal EHR requirements).

C.11.6.2. The Contractor shall reimburse agents dispensed through these pharmacies as directed by the Government. These pharmacies will be paid the submitted costs plus a dispensing fee negotiated by the Government. Dispensing fees may be updated annually, as directed by the Government.

C.11.6.3. The Contractor shall exclude these pharmacies from the measurement of network access standards and the calculation of the Retail Network Cost Control or Retail Network Specialty Cost

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Control incentives in Section H.

C.11.7. Federal EHR

C.11.7.1. The Contractor shall connect to the Federal EHR through use of a commercial switch. The Contractor shall accept pharmacy claims from all MTFs using the Federal EHR, including US Coast Guard sites.

C.11.7.1.1. The Contractor shall process claims received through the Federal EHR in accordance with C.5.4.

C.11.7.1.2. The Contractor shall support testing of the Federal EHR in the environments described in C.11.14.3.

C.11.7.1.3. The Pharmacy Contractor shall submit an ICD for the Federal EHR interface in coordination with the Federal EHR contractor.

C.11.7.1.4. The Contractor shall receive a supplemental file from the Federal EHR contractor containing claims that could not be transmitted through the usual adjudication process. The Contractor shall post these claims to the patient profile, to be available for use in edits related to future claims adjudication, transmit the transactions to the PDW, and update the PDMP in accordance with C.8.12.3.2.

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C.11.7.1.4.1. The following shall be determined by mutual agreement between the Contractor and the Federal EHR Contractor, with Government concurrence:

- File format – Shall contain a minimum all data elements required to post the claims to the patient profile and flow to downstream processes, including the PDW and PDMP.
- File delivery – The Contractor shall provide a secure location to receive the file, coordinated with the DHMSM Contractor.
- File frequency – The file will be received no less than daily.

C.11.7.1.4.2. The details of C.11.7.1.4.1 above shall be documented in the Supplemental file specification. This document shall also capture any relevant process details including but not limited to any validation processes used to post claims to the patient profile.

C.11.7.1.4.3. In the event that a scheduled file cannot be transmitted due to system outage or other system issue, the Contractor shall work with the Federal EHR Contractor to ensure delivery of all missing transactions. The contractor shall then ensure the PDW and PDMP are brought up to date as soon as possible.

C.11.7. Pharmacy Data Warehouse Interface

C.11.7.1. The Contractor shall develop and maintain an interface to the Pharmacy Data Warehouse (PDW). The PDW is the Government-owned system of record administered by the PDW contractor for the DoD Pharmacy Program. It contains detailed data for every transaction from all POS, as well as extensive reference data to assist in the categorization and aggregation of agents, beneficiaries, prescribers, pharmacies, and associated prescription costs. PDW supports all aspects of DoD reporting requirements, data mining, ad hoc queries, and research.

C.11.7.1.4. The Contractor shall obtain data from DMDC and any other sources required to supplement information that comes cross on the claim.

C.11.7.2. After award the Contractor shall provide a data exchange consistent with the Pharmacy Warehouse Data Dictionary and Data Schema. The pharmacy warehouse data feed shall be capable of transmitting new and updated data. Data feeds shall be provided on a daily basis based on the tech spec provided by the Government. The Contractor shall ensure that all paid, rejected, and reversed transactions from all POS, including reimbursement claims, and their required data elements are transferred to the PDW. The content of the PDW evolves with significant changes, including but not limited to the implementation of new NCPDP standards and DoD Benefit Design changes. The Contractor shall coordinate such changes with the Government and support changes in the file feed and format to support the changes.

C.11.7.3. In the event that a daily file cannot be transmitted due to system outage or other system issue, the Contractor shall work with the Government to ensure that the PDW is brought up to date as soon as possible.

C.11.7.4. In instances when the Government identifies inconsistent or missing information between the Contractor's system and the PDW, the Contractor shall correct the inconsistency, such as adding the data element to the PDW and providing missing or updated data. The Contractor will provide a plan with a timeline in collaboration with the Government and provide updates until resolved.

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C.11.7.5. The Contractor shall maintain an MOU with the PDW Contractor (CDRL A021).

C.11.8. Operational Medical Data Store (OMDS)

C.11.9. The Contractor shall connect to OMDS to receive claims for prescriptions dispensed in theater. Requirements TBD.

C.11.10. Immunization Tracking System (HL7IMM) Interface

C.11.10.1. The Contractor shall develop, maintain, and update an interface with the Immunization Tracking System, which maintains the immunization records of active duty and uniformed beneficiaries. The interface shall support Secured File Transfer Protocol (SFTP). The file format transmitted by the Contractor is based upon industry standard (HL7IMM) and contains retail immunization data. The frequency of the process shall be configurable; currently this is a daily process.

C.11.11. Forensic Toxicology Drug Testing Laboratory Information Management System

C.11.11.1. The Forensic Toxicology Drug Testing Laboratory (FTDTL) receives specimens collected from ADSMs and tested for selected compounds at regional labs. Results are aggregated centrally by LIMS (Laboratory Information System).

C.11.11.2. The Contractor shall develop, maintain, and update an interface between the Contractor and the LIMS in support of the examination of a beneficiary record for submitted chemical compounds. The interface shall support Secured File Transfer Protocol (SFTP). The flat file transmitted by the Contractor will contain response information to the query file received from LIMS. The frequency (e.g., 15 minutes) of the query process shall be configurable.

C.11.11.2.4. The Contractor shall provide the SFTP server required to support this interface.

C.11.12. TEDS

C.11.12.1. The Contractor shall connect to the TED System in accordance with the TSM, Chapter 2.

C.11.12.2. The Contractor shall support a connection to the TEDS test environment and submission of test files.

C.11.13. Testing Requirements

C.11.13.1. The Contractor shall support all testing and validating activities required to connect to DoD systems in accordance with current DoD policy.

C.11.13.2. The Contractor shall support test set-up, execution, and troubleshooting activities for sustainment of interfaces on a schedule that is mutually agreed on by all relevant parties. The Contractor may, on occasion, be required to provide test files, transaction data, and other documentation to other parties to be used in the validation of data transmitted between systems.

C.11.13.3. Test Environments. The Contractor shall develop and support test environments. At minimum, the Contractor shall provide, as a minimum, two environments:

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C.11.13.3.4. A production equivalent test environment. This environment shall reflect the current production capabilities, or, for new interfaces, the most developed version of those capabilities.

C.11.13.3.5. A development test environment. This environment shall include capabilities undergoing active quality assurance testing with external interfaces. The test environments shall support and be used for sustainment testing of current systems.

C.11.13.3.6. Test environments shall be made available for use by the government interface testing partners responsible for transition and sustainment of systems that support this contract.

C.11.13.3.7. At a minimum, environments shall be available during standard business hours in all CONUS time zones. Extended hours will be honored on a mutually agreed-upon schedule.

C.11.13.3.8. The test environments shall support connections by commercial entities and via the B2B Gateway.

C.11.13.3.9. The Contractor shall support concurrent connections from multiple testing partners.

C.11.13.3.10. The Contractor will not exchange PHI/PII with external partners as test data. Prescriber name and ID shall be masked on all outgoing transactions to testing partners. Test data files may be excluded from this masking if the Government, Contractor, and external partners agree that to do so would compromise testing outcomes. All beneficiary profiles will be test beneficiaries in the current DMDC contractor test environment.

C.11.13.3.11. The Contractor shall make available a subset of test patients to each testing partner to use within the environment and those patients will not be used by the Contractor or any other interface partners unless mutually agreed for joint testing.

C.11.13.3.12. During the building of the test environment, the Contractor shall support setup and testing with external contractors on a mutually agreed upon schedule.

C.11.14. Electronic Claims Processing and Benefit Tools

C.11.14.1. The Contractor shall support commercial standard electronic tools in support of a range of activities in conjunction with ordering, renewing and filling outpatient prescriptions and communicating with prescribers. The tools shall support such functions including but not limited to entering and transmitting prescription orders, accessing formulary information, providing patient specific benefit information, providing patient prescription history, completing prior authorizations, and using direct messaging to engage with prescribers.

C.11.14.2. New tools and enhancements to existing tools shall be offered to the Government when they are made available to commercial clients.

C.11.14.3. All information provided using the electronic tools described in this section shall be accurate, complete, and timely, reflecting the current patient profile, benefit design, and other information.

C.11.14.4. The Contractor shall support e-prescribing for the retail network, MTFs, and TMOP, in accordance with current commercial standards. The Contractor shall manage and publish all data files and support real-time queries required to support all aspects of

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commercial e-prescribing practices. Commercial standards include: NCPDP SCRIPT, Real Time Prescription Benefit, and Formulary and Benefit standards.

C.11.14.5. The Contractor shall create, maintain, and publish accurate and up-to-date formulary information in all formats required to support widely adopted capabilities of commercial EHRs and all standard e-prescribing models.

C.11.14.6. The Contractor shall utilize tools to evaluate and maintain the accuracy of the formulary and benefit information in whatever format it is provided, including relevant patient, claims history, and pharmacy information, as required.

C.11.14.7. The Contractor shall ensure that all formulary and benefit information remains consistent across all the different ways in which the information is communicated. The Contractor shall have processes in place to quickly resolve any discrepancies, both those identified through the Contractor's monitoring and reported by prescribers and other users.

C.11.14.8. The Contractor shall also coordinate with all relevant stakeholders to ensure that information used by the Federal EHR is accurate and that the format is optimized for that system. The Contractor shall work with the Government, Federal EHR contractor, and any other party as required to resolve any issues related to the presentation of formulary information.

C.11.14.9. The Contractor shall support at least one electronic prior authorization (ePA) tool that meets the following minimum capabilities:

- Supports both submission and real-time adjudication of PAs and MNs
- Accessible to MTF prescribers worldwide
- Supports benefit design criteria and restrictions specific to a point of service
- Offers a clinical review based on the TRICARE benefit for beneficiaries with OHI
- Must support anyone who receives care at an MTF worldwide (e.g., direct care only or patients without DEERS eligibility)
- Any authentication process must be DoD Approved and not require use of a personal device
- Any terms and conditions required to use this platform shall be provided to the Government for review and concurrence
- Supports MTF prescribers without an NPI (a pseudo NPI)
- Accessible as a stand-alone platform via a website; may also be available through integration into the prescriber's EHR

If multiple ePA platforms are supported, only one has to meet these requirements.

C.11.14.10. The Contractor shall maintain and update plan participation status files with commercial e-prescribing hub(s), to allow prescribers' EHRs to associate beneficiaries with the TRICARE benefit. The plan participation file shall include all beneficiaries covered under this contract for which the Contractor has processed claims but will be limited to the minimum data fields required by the commercial e-prescribing hub to determine the appropriate formulary. The data fields submitted to identify TRICARE beneficiaries will be mutually determined between the Government, the Contractor, and e-prescribing hub and may vary from those used by most commercial plans. The

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Contractor shall also have a process to remove patients from the file after determining that they are no longer eligible for the TRICARE benefit.

C.11.15. Contractor Claims Data

C.11.15.1. The Contractor shall provide the Government with online access to the Contractor's system that stores TRICARE claims processing information, in order to facilitate responses to beneficiary issues, monitor MTF claims processing, and perform audits. The Government shall have the ability to look-up individual patient claims transactions and rejects and generate ad hoc reporting.

C.11.15.2. The Contractor shall provide access for up to 100 government personnel in multiple locations, as specified by the Government, through a web-based tool. This Contractor's database is to include all claim information and clearly differentiate between different claims types: Retail, TMOP, MTF, Non-DoD Agency, reimbursement claims, and Theater. The data shall include, at minimum, PA and MN authorizations, OHI status and records, benefit restriction authorizations, documentation of beneficiary support and services, and up-to-date, real-time claim details regarding prescription information, cost data, beneficiary demographics, prescriber, and dispensing pharmacy data.

C.11.15.3. All claim look-up data must be current, accurate, complete, and accessible immediately (same day). System must also include the ability to look up current claims by agent name or NDC. Data shall be retrievable for ad hoc reporting one day (24 hours). The Contractor shall provide training, a user guide, and ongoing customer support for this access. Training shall be provided as necessary to new users and when there are significant changes to the Contractor's system.

C.11.16. Interfaces with TRICARE Medical Contractors.

C.11.16.1. At the Government's direction, the Contractor shall provide data to and accept data from the TRICARE Medical Contractors ("Medical Contractors") and the Government.

C.11.16.1.1. The Contractor shall collaborate with the Medical Contractors and the Government in evaluating cost and clinical effectiveness of specific aspects of the pharmacy benefit, including but not limited to specialty agents (C.4.1) and home infusion therapy programs.

C.11.16.1.2. The Contractor shall coordinate with Medical Contractors to obtain data required to support outcomes measurement and other value-based initiatives. Data to be exchanged could include, but is not limited to, ICD codes, hospital admissions data, and laboratory values.

C.11.16.2. The TRICARE Medical Benefit includes health care plans that manage their own pharmacy claims. The Contractor shall accept batch claims files of these pharmacy claims for application to the central patient profile and transmission to the pharmacy data warehouse. The Contractor shall not reject these claims for benefit design or other edits otherwise applied in claims adjudication. The formatting and timing of the data files will be determined by mutual agreement between the Contractor, the medical contractor(s) and the Government.

C.11.16.3. In the event that a file cannot be transmitted due to system outage or other system issue, the Contractor shall work with the medical contractor(s) to ensure that the file is sent as soon as possible.

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C.11.16.4. If the medical contractor(s) provide inaccurate or incomplete information to the Pharmacy contractor(s), the Contractor shall work with the medical contractor(s) to update missing and update data and apply it soon as possible.

C.11.16.5. In the event that individual claims cannot be posted to the patient profile, the Contractor shall attempt to correct the claim and work with the provider of the file to validate any required information.

C.11.16.6. The Contractor shall establish interfaces with the Medical Contractors for purposes of providing patient agent history. This interface shall utilize HL7 FHIR or a comparable commercial standard with widespread adoption by medical payers. The Contractor shall, no less than weekly, provide a data file containing pharmacy claims for all beneficiaries enrolled to that medical contractor. Beneficiaries will be identified by means of a data file provided by each Medical Contractor, to be refreshed no more frequently than weekly. Data shall be provided in a commercial standard format.

C.12. Privacy, HIPAA, and Federal Information Requirements

The Contractor shall comply with all requirements in the TRICARE Manuals and as documented in the requirements located on the website referenced in Attachment J-3, line 13. The relevant areas covered include:

- Protection of PHI and PII
- Records Management
- Freedom of Information Act
- System of Records
- Data Sharing Agreements
- Privacy Act and HIPAA Training
- HIPAA Business Associate Provisions
- Breach Response

C.12.1. The Contractor shall ensure that it does not use or disclose PHI or PII received for DVA or DoD beneficiaries in any way that will remove or transfer the PHI/PII from a jurisdiction subject to the laws of the United States. The Contractor shall not release Government data without approval by the CO or COR.

C.13. Financial

C.13.1. The Contractor shall not collect, receive, retain, or benefit from, directly or indirectly, any additional fees, rebates, discounts, premiums, or any other consideration of any kind specific to processing TRICARE prescriptions other than recoveries (payable to the U.S. Treasury) resulting from audits of network pharmacies. The Contractor shall not negotiate or collect, receive, retain, or benefit from, directly or indirectly, any agent rebates, data-use rebates, vendor chargebacks, or any other like consideration of any type from agent manufacturers, wholesalers, and/or network pharmacies on behalf of the Government or for itself in regard to the services performed under this contract. The prohibitions cited herein against earning an additional fee and negotiating with pharmaceutical agent manufacturers or wholesalers do not apply to the task(s) described in C.6.8.28. The Government may also apply other exceptions to support initiatives determined to be in the Government's best interests.

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C.13.2. Recoupments

C.13.2.1. The Contractor shall recoup Government funds and funds not properly collected at the time the prescription was dispensed in accordance with TOM, Chapter 10. Prescriptions subject to recoupment may be identified by the Government, or by the Contractor through its audit procedures. The Contractor is responsible to assist the Government in recouping any previous overpayment made to a provider by the TRICARE Pharmacy Benefits Program. This requirement is not limited by the dates of service under this contract. This does not apply to the collection of debts resulting from the Contractor granting credit to beneficiaries under C.6.3.5. Such debts are not owed to the Government. Therefore, the Contractor's collection of unpaid cost-sharing amounts is at the Contractor's own risk utilizing practices separate and apart from any recoupment procedures under this contract.

C.13.2.2. The Contractor shall recoup from patients with retroactive eligibility changes identified by DMDC. These recoupments shall be reported under CDRL Q130.

C.13.3. TED Submission Requirements

C.13.3.1. The Contractor shall submit a TED record for each prescription processed to completion and each completed Clinical Review, in accordance with TSM, Chapter 2, and the TOM, Chapter 1. MTF claims (See C.5) and rejected electronic claims are excluded. Adjustments, cancellations, or corrections to TED records shall be made as required to ensure financial transactions are complete and correctly recorded in TED records by fiscal year and by bank account (i.e., Medicare Dual eligible or TRICARE only).

C.13.3.2. The Contractor must be able to adjust prior Contractors' TEDs as necessary.

C.13.3.3. The Contractor shall have the ability to make adjustments to TED records without creating inaccuracies in the clinical record.

C.13.3.4. For electronic retail claims, the Contractor may hold the TED for 10 days to allow for reversals of non-complaint prescriptions (C.3.1.9). Claims reversed or cancelled within the 10 day hold period do not require a TED. Reversals processed after the date the TED was submitted will require an adjusted or cancelled TED record. All other claims must submit TEDs in accordance with the TOM, Chapter 1.

C.13.3.5. The accuracy rate for TED edits shall not be less than:

C.13.3.5.1. 95% after six months of performance during the first option period; and

C.13.3.5.2. 99% after nine months and thereafter during the entire term of the contract.

C.13.3.6. The Contractor shall provide reporting to the Government on TEDs processed for all relevant CLINs (CDRL Q090).

C.14. Management

C.14.1. Quality Management

C.14.1.1. The Contractor shall provide a comprehensive Quality Management Plan (A050). The plan

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shall be reviewed and revised no less than annually or when changes are required.

C.14.1.1.1. The Contractor's plan shall incorporate a proactive and industry-leading quality management approach for all functions and requirements under this contract. The Contractor's approach shall establish quality controls that meet the requirements in TOM Chapter 1, Section 4; TOM Chapter 23, Section 4, including reporting requirements (CDRL Q140 and CDRL M090).

C.14.1.1.2. The Contractor's plan shall address at least the following: pre-release testing processes, process mapping for all cases, self-inspection plans, continuous surveillance, continuous quality monitoring, continuous improvement, methods for prompt reporting, and methods for root cause analysis.

C.14.1.1.3. The Contractor's approach to quality management shall include the following:

- Promote proactive identification of potential issues, errors, and defects before they impact beneficiaries and other stakeholders
- Inform the Government within one business day of when an issue, error, or defect is identified
- Take available steps to mitigate the impact of the issue, error or defect
- Provide a corrective action plan and update the Government on a mutually agreed schedule until the issue is resolved, including all corrections
- Provide a long-term resolution to the issue and perform any required corrections
- Develop and implement procedures to ensure non-recurrence of issues, errors, and defects.

C.14.2. Contractor Training and Identification

C.14.2.1. The Contractor shall ensure that its staff and any subcontractors are thoroughly trained and knowledgeable regarding the requirements of this contract.

C.14.2.2. Contractor personnel and subcontractors shall identify themselves as contractors or subcontractors during meetings, telephone conversations, in electronic messages, or correspondence related to this contract.

C.14.2.3. Contractor personnel shall be clearly identifiable while on Government property by wearing appropriate badges.

C.14.2.4. Contractor personnel, while performing in a contractor capacity, shall not use their retired or reserve component military rank or title in any written or verbal communications associated with the contract under which they provide services.

C.14.2.5. The Contractor shall avoid any action or statement that is inconsistent with the requirements of this contract.

C.14.3. The Contractor shall provide to the CO an updated management organizational chart identifying key personnel at the post-award conference. The Contractor shall notify the Government in advance of any changes to key personnel and provide an updated organizational chart to reflect the change. Updated organizational charts shall also be provided

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in the event of a change in the management structure. The Contractor shall monitor and log operational issues and provide updates of this log for recurring meetings with the Government (CDRL W040).

C.14.4. The Contractor shall monitor performance across all major areas of performance under the contract in accordance with CDRL M010.

C.14.5. The Contractor shall coordinate with the CO and COR no less than five business days prior to meeting with any DoD personnel, including the Military Services or Government Contractors in support of DoD. Such coordination will include sharing a list of attendees and tentative agenda of the topics to be discussed. The Contractor shall also provide an after-action summary/minutes documenting what was discussed and any materials provided by the Contractor.

C.14.6. No less than quarterly, the Contractor shall plan and conduct meetings with the Government to review progress and status of activities under the Contract. Meetings shall be held at a mutually agreed upon location. The final agenda and meeting materials shall be provided no less than five business days in advance of the meeting.

C.14.7. Within five business days after any meeting with the Government, the Contractor shall provide minutes and action items to the COR.

C.14.7.1. The Contractor shall provide the Government with licenses to Medispan and First Data Bank (FDB) for purposes of calculating the Cost Control Incentives (see Section H). At a minimum, access to the FDB Core Package should be included. The Government anticipates up to 60 users for FDB and 10 users for Medispan.

C.14.8. The Contractor shall provide a version of the contract that is releasable under the Freedom of Information Act in accordance with CDRL A010.

C.14.9. The Contractor shall maintain standard operating procedures (SOP) documents for areas of contract performance that may have complex and variable processes that need to be communicated to stakeholders. SOPs should include, but are not limited to: Emergency Declarations, Drug Recalls, Deployed Prescription Program, Prescription Monitoring Program, Other Health Insurance, PDMP, and Beneficiary Communication Templates. SOPs shall be developed during transition, in collaboration with the Government, and shall be added and updated as necessary throughout the period of performance. SOPs shall be made available to the Government in accordance with C.14.13.

C.14.10. Continuity of Operations

The Contractor shall develop a Continuity of Operations Plan (COOP) that meets the requirements in TSM, Chapter 1, Section 1.1. Additionally, the plan shall address how all contract services will be restored after an emergency event. This includes both situations of short or long duration and disruptions internal and external to the Contractor. The COOP shall be written to meet all performance standards established in this contract, including the restoration of critical functions within 24 hours. At minimum, critical functions include real time claims processing. The COOP shall be delivered to the Government prior to the start of pharmacy services. The plan shall be reviewed annually and an updated version provided to the

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Government prior to the start of each subsequent option period (CDRL A030). The disaster recovery plan established in the COOP shall also be tested and results provided to the Government in accordance with the requirements established in the TSM (CDRL A031).

C.14.11. Clinical Support Agreements. Clinical Support Agreements (CSAs) may be used to optimize MTF pharmacies, as described in TOM, Chapter 15, Section 3. The CO will incorporate CSAs via bilateral modification.

C.14.12. At the request of the Government, the Contractor shall provide additional reports to support benefit design review and evaluation. The Contractor shall deliver these results in the format and method specified by the Government (CDRL R030).

C.14.13. Contractor Maintained Documents. The Contractor shall maintain an online repository that is readily accessible to identified government users for purposes of contract oversight. In this location, the Contractor shall maintain current and archived versions of SOPs, technical documentation (including interface control documents, payer sheets, etc.), benefit design documents and agent lists, beneficiary communication templates (including updated MHS Style Guide and TRICARE Style Guide and previously approved deliverables), training documents,

and other item listed in this contract or as otherwise requested. All documents shall include the date they were most recently updated. Archived versions shall include effective dates and be clearly distinguishable from the versions currently in use. For public-facing beneficiary and provider documents, final versions within the online repository shall be made readily available and distinguishable for the Government's use through a logical, hierarchical data structure.

C.15. Contract Transition

C.15.1. Transition-In.

C.15.1.1. Contract transition-in shall be conducted in accordance with the TOM, Chapter 23 and the following.

C.15.1.2. The Contractor shall complete all transition-in efforts in accordance with the Transition-In Plan (CDRL T010) and be prepared to begin delivery of services in accordance with Section F of this contract. Transition-in efforts shall be completed prior to the applicable start of pharmacy services under this contract and shall include:

- Connectivity to all required government systems.
- Complete testing and certification that development is complete and systems are functional for successful interaction with all required government systems.
- Successful completion of integration, benchmark, and stress testing for all systems. Initial testing shall include but is not limited to all required financial transactions such as tracking transactions by fiscal year, voided, stale-dated or reissued checks, adjustment and cancellation TEDs, and recording and reporting collections. Significant issues experienced in testing may require that the Contractor repeat the tests to confirm that the appropriate corrections are in place. Exit criteria will be determined by the Government. The COR or other designated authority will certify the successful completion of integration, stress, and benchmark testing.
- Successful load of Automated Data Processing (ADP) and any other data from the

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Outgoing Contractor or DHA, including but not limited to:

- TRICARE Encounter Provider Files (TEPRV)
- PA/MN Files
- TMOP Open Refill File Prescription Monitoring Program Restrictions
- Data in support of Deployed Prescription Program
- Program Integrity Files

C.15.1.3. In addition to the requirements in TOM Chapter 23, Section 5, the Contractor's transition-in plan shall include the following:

- An overview of requirements
- Critical path and milestones
- Identification of risks, both overall and with reference to specific requirements and milestones. The plan shall include risks classified as moderate or above and their categorization. The Government may identify additional risks for the Contractor to address in their plan.
- Risk management strategies to be applied.

C.15.1.4. Throughout transition, the Contractor shall provide weekly reports on the status of transition relative to the transition plan (CDRL T020).

C.15.1.5. The Contractor shall arrange/attend meetings with the Government and/or external agencies in support of all requirements under this contract, including the establishment of all systems interfaces necessary to meet the requirements of the contract including but not limited to the PDW, DMDC, DHA/TEDS, MTFs, theater medical data, and DLA/NPV. This will include integration testing meetings on each business day during transition-in or as otherwise directed by the Government, beginning at a date determined by the Government.

C.15.1.6. During contract transition-in, the Contractor shall provide estimates to DMDC of projected DEERS query volume over the period of performance. These estimates shall be provided in accordance with CDRL A090.

C.15.1.7. The Contractor shall complete the required B2B questionnaires during transition and shall obtain Government approval prior to receiving authorization to connect to any Government system (including test systems and environments). (See C.11.4)

C.15.1.8. The Contractor shall support all required testing activities during the transition period (See C.11.14).

C.15.1.9. At the Transition meeting between the incoming and outgoing Contractors and the Government representatives, contractors shall mutually agree to a run-off schedule and file formats for all processes, including but not limited to clinical reviews, paper claims, TMOP prescription processing and beneficiary correspondence.

The Contractor shall retain and use the TRICARE Encounter Provider record (TEPRV) provider numbers previously established by the outgoing Contractor for all TED submissions (TSM, Chapter 2, Section 1.2).

C.15.1.10. As part of transition efforts, the Contractor shall provide formulary support materials as

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described in C.7.2.1 in support of the first P&T meeting after go-live.

C.15.1.11. The Contractor shall review its proposed OHI process with the Government during contract transition-in (see C.2.9).

C.15.1.12. Performance-Based Transition-In Milestones. Successful completion of a performance-based transition-in milestone consists of the Contractor meeting the stated objectives established and verification by the Government. These milestones and their completion criteria are listed below. Achievement of each transition-in milestone triggers a performance-based payment comprising a percentage of the total transition-in price, as described in Section G.

Milestone	Completion Criteria
Approach to DPP	Acceptance of technical approach by Government
Approach to OHI	Acceptance of technical approach by Government
Approach to TMOP	Acceptance of technical approach by Government
B2B Questionnaire - Initial	Approved; covers at minimum DEERS and TEDS
B2B Questionnaire - Final	Approved; covers all required interfaces
Benchmark Testing	Completion of benchmark testing, with all findings addressed
DEERS Volume Estimates	Acceptance of Estimates and successful testing of PDW file
Formulary Search Tool Mock-up Previewed	Gov't reviews & validates it meets requirements
MOU with DCPAD	Fully executed MOU
MTF Help Desk SOPs	Accepted by the Government
NIST Checklist	Submission of complete System Security Plan and checklist, certifying NIST compliance (See Section H) as well as confirmation that Basic Assessment has been posted to Supplier Performance Risk System (SPRS) (See DFARS 252.204-7020)
Organizational Conflict of Interest (OCI) Plan	Approval by the Government
Post Award Conference/Transition	Completion of meeting
Quality Management Plan	Acceptance of plan; meets requirements
Prescription Drug Monitoring Program	Successfully connection to PMP Interconnect and stand-up of MHS "State"
Run-off and/or Cutover Plans	Government acceptance of plans
Website Mock-up previewed	Approval by Government; meets requirements

Transition-In Mailings

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C.15.1.12.1. The Contractor shall, in coordination with the outgoing Contractor, identify beneficiaries who, during the six months prior to the letter mailing date, used pharmacies that are not in the Contractor's pharmacy network. The incoming Contractor will inform these beneficiaries by letter that the pharmacy they previously used is no longer in the retail pharmacy network and provide the 3 closest network pharmacies by drive time and link to the pharmacy locator tool. This letter will be mailed so that beneficiaries will receive it 30 to 40 days prior to the start of pharmacy services.

C.15.1.12.2. The Contractor shall mail notices to beneficiaries who have filled prescriptions at TMOP or a retail pharmacy, during the six months prior to the mailing date. The letter shall include, at a minimum, the information required under the notice in C.9.3.2.

C.15.1.12.3. At the direction of the Government, the incoming Contractor shall mail letters to beneficiaries identified by the Government to communicate changes to the benefit during contract transition-in.

C.15.1.12.4. The incoming Contractor shall establish MOUs with internal and external DHA partners at the direction of the Government, or as the Government or Contractor otherwise deems necessary to meet the requirements of the contract, including necessary cooperation, system interfaces, exchange of information, and POCs for such things as program integrity issues, case management (including coordination of care for patients who receive specialty agents (C.4.1) or home infusion services), third-party liability and claims jurisdiction issues. All MOUs are subject to annual review and update, as required, by the Contractor. At minimum, the Contractor shall establish MOU with the following:

- DHA Public Affairs Division (CDRL A020)
- DHA Managed Care Support Contractors (CDRLs A023 & A024)
- DHA Compliance Review Contractor (CDRL A025)
- Transition-In MOU (CDRL R120)
- DHA Information Operations (Pharmacy Data Warehouse) (CDRL A021)
- DHMSM/Federal EHR Contractor (CDRL A022)

The Contractor will notify the Government of any additional MOUs that are required so numbers may be assigned for submission of deliverables.

C.15.2. Contract Transition-Out

C.15.2.1. The Contractor shall complete contract transition-out in accordance with the TOM, Chapter 23, and the following activities.

Upon award of any subsequent contract, the Contractor shall support transition activities of the incoming Contractor with minimal disruption of services to the beneficiaries. The Outgoing Contractor shall execute an MOU with the Incoming Contractor. All points of beneficiary contact shall provide outgoing messaging with the Incoming Contractor's contact information for no fewer than 45 days after the start of service delivery under the successor contract.

C.15.2.2. The Outgoing Contractor shall provide the Incoming Contractor with all applicable versions of the benefit design and associated agent list (see C.7.3), with effective dates for the final two years of this contract. The Outgoing Contractor shall also provide an OHI data

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file.

C.15.2.3. The Contractor shall submit a Transition-Out Plan (CDRL T030) and regular status reports (CDRL T040).

C.15.2.4. The Contractor shall maintain sufficient qualified staff to meet all requirements of the contract, including beneficiary services and final processing of all postponing claims including TED reporting requirements.

C.15.2.5. The Contractor's transition-out activities will be coordinated through the Government.

C.15.2.6. The Outgoing Contractor shall send a notice to all eligible beneficiaries who have used pharmacy services in the previous 12 months. The Government shall provide the new Contractor's information and POC to the Outgoing Contractor at least 120 calendar days prior to the end of the final option period of this contract. The notice will provide the new Contractor's information and points of contact (mailing addresses, email addresses, and phone numbers). The notice shall be sent 95 calendar days prior to the end of this contract with intent of delivery by X date.

(End of Section C)